

DISCLOSURE REPORT | DATED MAY 23, 2025

THE NEW YORK AND PRESBYTERIAN HOSPITAL

For the Three Months Ended
March 31, 2025





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FORWARD-LOOKING STATEMENTS

Certain statements included or incorporated by reference in this Disclosure Report constitute projections or estimates of future events, generally known as forward-looking statements. These forward-looking statements are based on the current plans and expectations of NYPH and are subject to known and unknown uncertainties and risks, many of which are beyond the control of NYPH, that could significantly affect current plans and expectations and NYPH's future financial position and results of operations. As a consequence, current plans, anticipated actions and future financial position and results of operations may differ from those expressed in any forward-looking statements made by or on behalf of NYPH. Investors are cautioned not to unduly rely on such forward-looking statements when evaluating the information presented herein. The achievement of certain results or other expectations contained in such forward-looking statements involves known and unknown risks, uncertainties and other factors that may cause actual results, performance or achievements described to be materially different from any future results, performance or achievements expressed or implied by these forward-looking statements. NYPH does not plan to issue any updates or revisions to those forward-looking statements if or when changes in its expectations, or events, conditions or circumstances on which such statements are based, occur.

PREVIOUSLY REPORTED INFORMATION WITH RESPECT TO PRIOR PERIODS

Due to changes in accounting standards and related guidance, the application of relevant accounting standards, or an event such as a merger, that require a reclassification or restatement of certain items, and to adjustments in utilization data that occur in the normal course of patient care or as services are billed and coded, certain financial information and utilization data set forth herein with respect to past periods may differ from what NYPH has previously reported in disclosure documents previously published to the Municipal Securities Rulemaking Board's Electronic Municipal Market Access (EMMA) website.

THIRD-PARTY STATISTICS AND DATA

To the extent this Disclosure Report includes statistics and other data relating to the healthcare industry in the United States that have been derived from third-party sources, such statistics and data are not necessarily reflective of current or future industry and market conditions. While NYPH has no reason to question the accuracy of such statistics and data, such statistics and data have not been independently verified by NYPH.





ORGANIZATIONAL STRUCTURE

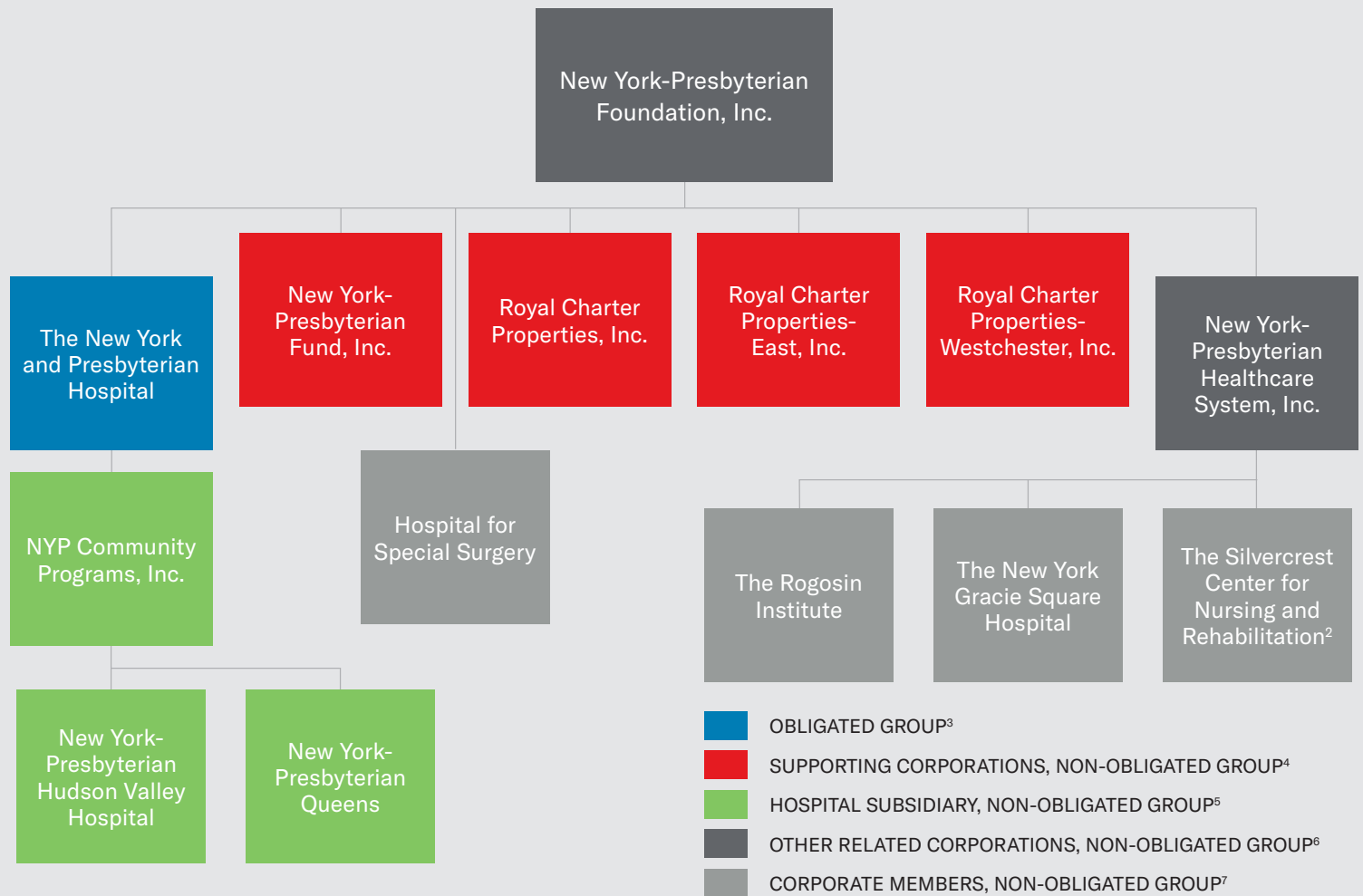
The New York and Presbyterian Hospital (“NYPH”), a New York not-for-profit corporation, serves as the academic and quaternary care hub of a network of health care providers, which, as of the date of this Disclosure Report, includes NewYork-Presbyterian/Queens (“NYP Queens”) and Hudson Valley Hospital Center (d/b/a NewYork-Presbyterian Hudson Valley Hospital) (“NYP Hudson Valley” and together with NYP Queens, the “Regional Hospitals”), and certain other entities, all located in New York. NYPH owns and operates inpatient facilities at eight campuses. Five of these campuses are in the New York City Borough of Manhattan and are referred to herein as “NYP/Weill Cornell,” “NYP/Columbia,” “NYP Lower Manhattan,” “NYP Morgan Stanley,” and “NYP Allen.” One of the campuses is in the Borough of Brooklyn and is referred to as “NYP Brooklyn Methodist.” The other two campuses are in Westchester County, New York and are referred to as “NYP Westchester” and “NYP Westchester Behavioral Health.” See, “NYPH Campuses” in this Disclosure Report. NYP Queens and NYP Hudson Valley each owns and operates an inpatient facility at its campus in, respectively, the Borough of Queens and Westchester County.

As used in this Disclosure Report, the term “NYPH” refers to The New York and Presbyterian Hospital as a legal entity separate from the other entities (the “Consolidated Entities”) that are part of the consolidated group that is reported in the consolidated financial statements of the Hospital (as defined below) prepared in accordance with U.S. generally accepted accounting principles. The Consolidated Entities include, among others, NYP Community Programs, Inc. (“NYP Community Programs”) and the Regional Hospitals. The term “the Hospital” is used in this Disclosure Report to refer to NYPH and its Consolidated Entities collectively.

New York-Presbyterian Foundation, Inc. (“NYPFI”) is an affiliate that is related to NYPH through the NYPH Board of Trustees. NYPH also is affiliated with several entities that support NYPH through fundraising and real estate holdings. These affiliated entities (“Supporting Corporations”) include: (i) New York-Presbyterian Fund, Inc. (“Fund, Inc.”), which solicits, receives, invests and administers philanthropic funds for NYPH, the Regional Hospitals, and other charitable organizations approved by the Board of Directors of Fund, Inc., both directly and through Hudson East River Systems, LLC (“HERS”), an affiliated investment entity, and (ii) three real estate holding companies: Royal Charter Properties, Inc., Royal Charter Properties-East, Inc., and Royal Charter Properties-Westchester, Inc. (collectively, the “RCP Corporations”). NYP Community Programs, a subsidiary of NYPH, serves as the parent entity of each of the Regional Hospitals. Another affiliated entity, NewYork-Presbyterian Healthcare System, Inc. (“NYPHSI”), serves as the corporate link to entities that are referred to herein as “Corporate Members.”

None of NYPFI, NYPHSI, the Supporting Corporations, the Regional Hospitals, the Corporate Members or the other affiliated entities have any obligations with respect to indebtedness of NYPH.

NEWYORK-PRESBYTERIAN ORGANIZATIONAL STRUCTURE¹



1. This NewYork-Presbyterian Organizational Structure chart does not include all entities which are included in the consolidated financial statements of The New York and Presbyterian Hospital (together with the Consolidated Entities (as defined below), the “Hospital”; as a legal entity separate from the Consolidated Entities, “NYPH”). Among others, it does not include professional corporations and other affiliates controlled by NYPH or one of the Regional Hospitals. For additional information regarding these entities, see Note 1 to the Hospital’s Unaudited Consolidated Financial Statements and Supplementary Information As of and For the Three Months Ended March 31, 2025, which are being posted on EMMA with this Disclosure Report.
2. The Silvercrest Center for Nursing and Rehabilitation has entered into an agreement to sell substantially all its assets. Closing of the sale is subject to the satisfaction of certain closing conditions, including receipt of the New York State Public Health and Health Planning Council and other regulatory approvals.
3. In connection with the issuance of its Series 2015 Bonds in February 2015, NYPH formed an Obligated Group under and as defined in its Master Trust Indenture. Currently, NYPH is the sole member of the Obligated Group; the Consolidated Entities are not members of the Obligated Group. For a discussion of the Master Trust Indenture, the Obligated Group and the debt covered thereby, see “Financial and Operating Information—Outstanding Long-Term Indebtedness—The MTI Indebtedness” on page 43 in this Disclosure Report.
4. For a discussion of the Supporting Corporations, see “ORGANIZATIONAL STRUCTURE” on page ii of this Disclosure Report.
5. Hospital subsidiaries are those entities under the control of NYPH or one of its direct subsidiaries. These entities are part of the Hospital’s consolidated group that is reported in the consolidated financial statements prepared in accordance with U.S. generally accepted accounting principles. Entities included in the Hospital’s consolidated financial statements, other than NYPH, are not legally obligated on NYPH’s indebtedness. The entities that are part of the Hospital’s consolidated audit group, other than NYPH, are referred to in this Disclosure Report as the “Consolidated Entities”.
6. These corporations provide a corporate or contractual link to certain entities. For additional details, see “ORGANIZATIONAL STRUCTURE” on page ii of this Disclosure Report.
7. For a discussion of the Corporate Members, see ““ORGANIZATIONAL STRUCTURE” on page ii of this Disclosure Report.

Note: NYPH is the only entity that is obligated to repay its indebtedness, including (i) The New York and Presbyterian Hospital Taxable Bonds, Series 2015, Series 2016, Series 2019 and Series 2020 (respectively, the “Series 2015 Bonds”, the “Series 2016 Bonds”, the “Series 2019 Bonds” and the “Series 2020 Bonds”) and (ii) the Dormitory Authority of the State of New York The New York and Presbyterian Hospital Obligated Group Revenue Bonds, Series 2023A (the “Series 2023 Bonds”). None of the assets or revenue of any of the other entities in the chart above is committed to the repayment of such indebtedness.



Hospital Campus



Medical Group



Ambulatory Care Network



Ambulatory Care Center



ColumbiaDoctors

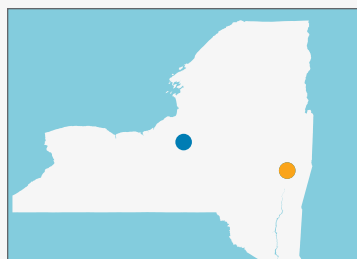


Weill Cornell Medicine

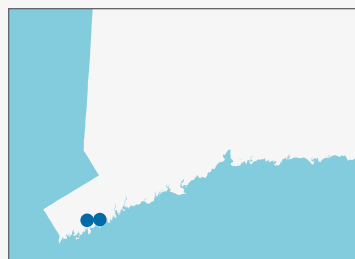


Corporate

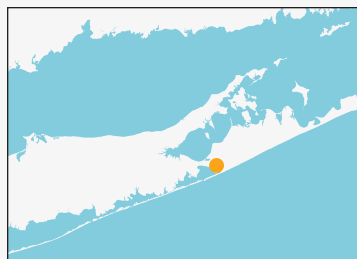
NewYork-
Presbyterian



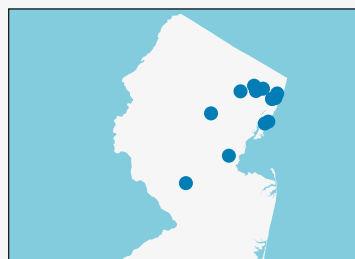
Upstate



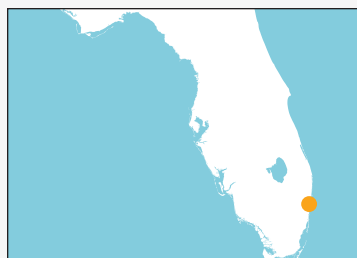
Connecticut



Long Island



New Jersey



Florida

NYPH, the Regional Hospital, the Medical Groups, ColumbiaDoctors and Weill Cornell Medicine collectively provide care at over 350 ambulatory care and ambulatory surgery centers, and primary and specialty care offices in New York City, Long Island and the Lower Hudson Valley, in addition to the ten inpatient facility campuses.

INTRODUCTION

General

NYPH, a New York not-for-profit corporation, was created as a result of the January 1998 merger of The Society of The New York Hospital, which was created in 1771 under a charter granted by King George III, and The Presbyterian Hospital in the City of New York. NYPH, which operates at eight campuses in Manhattan, Brooklyn and Westchester County, New York, was recognized as one of the best hospitals in the United States and a top hospital in New York by the 2024-2025 *U.S. News and World Report* Best Hospitals Honor Roll.

NYPH ranks in the top five nationwide in seven specialties: cardiology, heart & vascular surgery (No. 5); neurology and neurosurgery (No. 4); orthopedics (No. 4); psychiatry (No. 3); diabetes and endocrinology (No. 2); urology (No. 2); and rheumatology (No. 3), a collaborative program with Hospital for Special Surgery. NYPH ranked in the top 20 in 14 of 15 specialties, more than any other hospital in New York.

NYPH was also recognized as the #1 children's hospital in New York, by *U.S. News & World Report's* 2024-2025 "Best Children's Hospitals" rankings and was the only hospital in New York to rank among the top 50 hospitals across all 11 pediatrics specialties evaluated.

NYPH is a leader in graduate medical education, serving as the primary clinical teaching facility for two of the leading medical colleges in the United States: The Joan and Sanford I. Weill Medical College of Cornell University ("Weill Cornell Medical School") and Columbia University Vagelos College of Physicians and Surgeons ("Columbia VP&S") (collectively, the "Medical Schools"). Through this collaboration, NYPH's patients have access to a full range of medical specialties, as well as some of the leading physicians in the United States.

Over the years, NYPH has developed its relationships with the Regional Hospitals (NYP Queens and NYP Hudson Valley) as part of its strategic goal of providing high quality, integrated care throughout the New York metropolitan area (the "New York Metropolitan Area"). Today, the doctors, nurses, and staff of NYPH and the Regional Hospitals provide care across the region at ten hospital campuses in New York City and its northern suburbs. Additionally, more than 180 primary and specialty care clinics and medical groups and an array of telemedicine services enable people across the New York Metropolitan Area to receive care in their own communities. NYPH has developed highly specific, patient-centered models of care to treat its diverse patient populations and has become the teaching hospital of one of the nation's largest and most comprehensive, integrated academic healthcare systems.

NYPH CAMPUSES

NYPH owns and operates one of the oldest hospitals in the nation. It has a history of more than 250 years of providing medical care in the New York Metropolitan Area, with seven of the eight campuses operated by NYPH having roots dating back more than 100 years. NYPH is a major academic medical center and currently has 3,402 certified inpatient beds.



NEWYORK-PRESBYTERIAN/WEILL CORNELL MEDICAL CENTER

NewYork-Presbyterian/Weill Cornell Medical Center (“NYP/Weill Cornell”) occupies the city blocks bounded by East 68th Street to East 71st Street and from York Avenue to the East River, together with various buildings in the area. Portions of the buildings that comprise the NYP/Weill Cornell campus are owned and/or occupied by Weill Cornell Medical School. NYPH has 879 certified inpatient beds at its facilities at the NYP/Weill Cornell campus, which includes 135 certified beds at the Alexandra Cohen’s Hospital for Women and Newborns (the “NYP Cohen Hospital”). The NYP Cohen Hospital, which is located on the top floors of the NYP David H. Koch Center (the “NYP Koch Center”), provides maternity, well-baby and neonatal intensive care unit services, and together with the NYP Koch Center houses a variety of outpatient services, such as radiation oncology, infusion, ambulatory surgery, endoscopy, pre-admission testing and other ancillary services.



NEWYORK-PRESBYTERIAN/COLUMBIA UNIVERSITY IRVING MEDICAL CENTER

NewYork-Presbyterian/Columbia University Irving Medical Center (“NYP/Columbia”) occupies the city blocks bounded by West 165th Street to West 168th Street and from Broadway to Riverside Drive, together with various other buildings in the area. Portions of the buildings that comprise NYP/Columbia are owned and/or occupied by Columbia University. NYPH has 738 certified inpatient beds at its facilities on this campus.



NEWYORK-PRESBYTERIAN MORGAN STANLEY CHILDREN'S HOSPITAL

NewYork-Presbyterian Morgan Stanley Children’s Hospital (“NYP Morgan Stanley”) is a pediatric acute care and ancillary services facility with 284 certified inpatient beds and is located on a site contiguous with the NYP/Columbia campus.



NEWYORK-PRESBYTERIAN WESTCHESTER BEHAVIORAL HEALTH CENTER

NewYork-Presbyterian Westchester Behavioral Health Center (“NYP Westchester Behavioral Health”), which is located in White Plains, New York, has 247 certified inpatient beds and provides inpatient and outpatient psychiatric and behavioral health services.



NEWYORK-PRESBYTERIAN LOWER MANHATTAN HOSPITAL

NewYork-Presbyterian Lower Manhattan Hospital (“NYP Lower Manhattan”) is located at 170 William Street, Manhattan, and is the only inpatient facility below 14th Street. It has 180 certified inpatient beds.



NEWYORK-PRESBYTERIAN ALLEN HOSPITAL

NewYork-Presbyterian Allen Hospital (“NYP Allen”) has 196 certified inpatient beds and is located at Broadway and the Harlem River in the Inwood section of Manhattan. It opened in 1988 under the sponsorship of The Presbyterian Hospital in the City of New York and offers acute care to residents of its service area in a community-based setting.



NEWYORK-PRESBYTERIAN WESTCHESTER (FORMERLY NEWYORK-PRESBYTERIAN LAWRENCE HOSPITAL)

NewYork-Presbyterian Westchester (“NYP Westchester”) has 288 certified inpatient beds and is located in Bronxville, Westchester County, New York.



NEWYORK-PRESBYTERIAN BROOKLYN METHODIST HOSPITAL

NewYork-Presbyterian Brooklyn Methodist Hospital (“NYP Brooklyn Methodist”) has 590 certified inpatient beds and is located in Brooklyn, New York.

RECENT DEVELOPMENTS

Sustaining Care Delivery Through Economic Headwinds

Like many healthcare organizations, NYPH and the Regional Hospitals are navigating current macroeconomic realities and anticipating challenges ahead. To support long-term sustainability, management is advancing growth initiatives and reviewing expenses across the system. As part of this process, NYPH and the Regional Hospitals are reducing their workforce by approximately 2%. Management believes this is a prudent step to ensure continued delivery of high-quality patient care and to maintain NYPH's commitment to the communities it serves.

First Fully Robotic Liver Transplant in New York Performed at NYP/Weill Cornell

Surgeons from NYP/Weill Cornell and NYP/Columbia performed the first robotic liver transplant in New York on April 1, 2025, helping advance the field of transplant surgery. This is one of only three programs in the United States and among a few centers worldwide that perform robotic liver transplants. The nine-hour procedure involved the surgeon operating a robot a few feet away from the patient, while looking through camera screens in the robotic console, using hands and feet to control instruments through small incisions. The team made five small holes in the abdomen to insert the robotic arms and a small incision in the lower abdomen to remove the recipient's liver and implant the donor's liver. This method allows for greater precision and reduces recovery time compared to traditional open surgery, which requires a large incision and longer hospitalization. Advancements in robotic liver surgery have enabled the team to undertake more complex operations and to improve outcomes for patients with liver cancer, living liver donors, and liver transplant recipients.



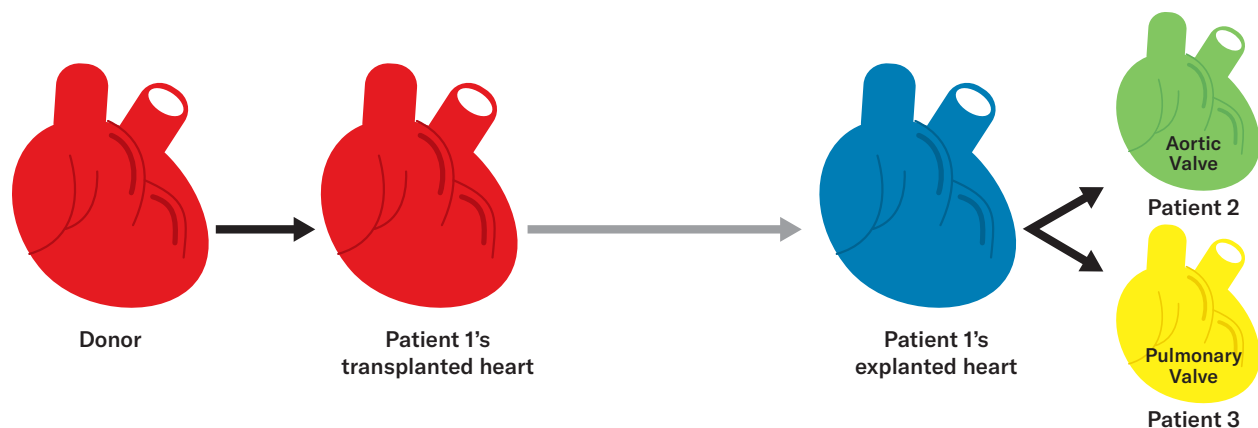
One Heart Saves Three Lives: Innovative Partial Heart Transplant Performed at NYP Morgan Stanley Children's Hospital



In a groundbreaking medical advancement, surgeons at NYP Morgan Stanley performed one of the first split-root domino partial heart transplants in the United States on July 25, 2024. This innovative procedure involved three young patients. One child, diagnosed with genetic cardiomyopathy, received a full heart transplant from a deceased donor. That child's healthy valves were then donated to two other children, who received heart valve transplants. This complex series of operations was executed by a multidisciplinary team.

The child with cardiomyopathy required a mechanical heart pump until a donor heart became available. That child's healthy pulmonary and aortic valves made the child an ideal candidate for this pioneering procedure. One of the recipients, born with truncus arteriosus, had undergone multiple surgeries. The other recipient, diagnosed with aortic stenosis, faced numerous surgeries to manage the patient's condition.

SPLIT-ROOT DOMINO HEART TRANSPLANT



These three back-to-back simultaneous procedures took 20 hours and involved meticulous logistical preparations, requiring two operating rooms, three intensive care unit beds, and coordination among multiple teams. Pediatric heart surgeons led the procedures, performing the full heart transplant, the aortic valve replacement, and the pulmonary valve transplant.

Teamwork and innovation were essential in achieving this medical feat. The successful transplants not only saved the lives of these three children but also paved the way for future advancements in pediatric heart treatments. The three patients recovered well, with the two valve recipients being discharged nine days later, and the full heart transplant recipient leaving the hospital about three weeks after the transplant. This procedure marks a significant step forward in the treatment of pediatric cardiovascular disease.

Transplant Program

Surgeons at NYP/Columbia and NYP/Weill Cornell performed 1,019 transplant surgeries during the twelve months ended December 31, 2024, and 239 transplant surgeries for the three months ended March 31, 2025. This included heart, lung, liver, kidney, intestine, and pancreas transplants from deceased donors, as well as living donor transplantation for kidney and liver. This was the most transplant surgeries performed by any hospital in the U.S. during each of these periods, according to the United Network for Organ Sharing (“UNOS”). NYPH been the leading transplant hospital in the nation for the past four years based on annual transplant volume.

HEART

NYPH’s adult transplant program began in 1971, based at the NYP/Columbia campus in Washington Heights where the first heart transplant at NYPH was completed in 1977. In October 2022, NYPH received approval from the New York State Department of Health to expand its renowned heart transplant program to the NYP/Weill Cornell campus on the Upper East Side. The first heart transplant at NYP/Weill Cornell was performed on February 1, 2023. Through March 31, 2025, NYPH has performed over 3,217 heart transplants.

LIVER

NYPH’s transplant program was the first in the nation to offer minimally invasive liver donor surgery and one of the first centers in the U.S. offering robotic donor surgery to all liver donors. Liver resection surgeries are also offered robotically.





KIDNEY

NYPH is recognized as the number one kidney transplant program by volume in the nation according to UNOS. As of March 31, 2025, surgeons at NYPH have performed over 11,486 kidney transplants. The program at NYP/Weill Cornell is the founding center of the National Kidney Registry, which is used in national efforts for paired donation, advanced kidney donation and exchange transplants. As of March 31, 2025, surgeons at NYP/Columbia and NYP/Weill Cornell performed over 6,400 living donor transplants (liver and kidney combined), the most of any hospital in the nation according to UNOS data. Of these, 5,854 were living donor kidney transplants and 554 were living donor liver transplants. NYP/Columbia is the lead investigator site in the PANORAMA trial, a clinical trial that aims to decrease the barriers to successful kidney transplantation. In addition, surgeons at NYP/Columbia have begun offering robotic surgery for kidney living donor surgeries and for kidney transplant surgeries.

LUNG

NYP/Columbia has one of the oldest lung transplant programs in the nation and is now the fourth largest lung transplant center by volume nationwide according to UNOS, performing more than 100 lung transplants annually. In 2024, 121 transplants were performed, including eleven lung-inclusive multiorgan transplants, marking the fourth consecutive year the number of lung transplants surpassed annual totals in prior years. Five-year and ten-year lung transplant survival rates for NYPH lung transplant patients exceed the national average, based on NYPH's internal analysis of UNOS data.



Quality

NYPH focuses on best practices in quality, safety and experience initiatives. In the aftermath of the COVID-19 pandemic, NYPH reengineered processes with a focus on building a strong, resilient workforce. This positive change has been reflected in the improved scores and new recognitions NYPH received regarding quality. NYPH's mortality index is at its lowest level and patient experience scores at their highest. NYPH also has top-tier rankings in other Vizient Quality indicators. Two campuses, NYP Brooklyn Methodist and NYP/Weill Cornell, won awards in September 2024 for their outstanding performance at the Vizient Connections Summit.

In 2024, for the second consecutive year, NYPH was awarded a Five-Star rating from the annual quality measurement program from the Centers for Medicare & Medicaid Services ("CMS"). This rating, the highest given, was received by fewer than 15% of hospitals nationwide. CMS stars reflect performance in safety of care, mortality, readmissions, patient experience, and the timeliness and effectiveness of care. NYPH's Five-Star rating has been driven by improvements in key patient safety indicators, including mortality, patient experience scores, and hospital-acquired infection rates, that were achieved by a focus on high-reliability and evidenced-based best practices.

According to data provided by CMS on February 19, 2025, NYPH ranks among the top 5 hospitals (at 99.8 percentile) nationwide for patient safety and adverse events (PSI 90). Additionally, NYPH and NYP Queens have earned an "A" Leapfrog Hospital Safety Grade, which places them among the top-performing hospitals nationally in terms of patient safety. NYPH and the Regional Hospitals are focused on maintaining a strong culture of safety and have numerous initiatives that are designed to prevent harm and promote safety in the delivery of care.

NewYork Quality Care—Accountable Care Organization (“ACO”)

NewYork Quality Care (“NYQC”), the ACO formed by NYPH and its affiliated Medical Schools, Columbia VP&S and Weill Cornell Medical School, earned shared savings in 2023 from CMS. NYQC generated over \$17.7 million in total savings. Of this amount, NYQC was awarded performance payments totaling \$6.9 million. NYQC serves about 39,165 attributed Medicare fee-for-service beneficiaries, mostly in the New York Metropolitan Area.

In addition to in-person visits, clinicians participating in NYQC used telehealth and remote patient monitoring to coordinate care, improve preventive services and reduce avoidable hospitalizations and emergency room visits. Nationwide, the Medicare Shared Savings Program includes more than 600,000 participating clinicians who provide care to eleven million Medicare beneficiaries. The program’s 453 participating ACOs generated \$5.2 billion in savings for Medicare in 2023, a net savings of \$2.1 billion to the Medicare program after accounting for performance payments, according to data from the National Association of Accountable Care Organizations.

Digital Transformation and Artificial Intelligence

NYPH is committed to leveraging technology and automation and investing in data science to deliver efficient operations, streamline clinical care, enhance patient, provider, and employee experience, and reduce the burden of human disease. Major initiatives underway that leverage technology include the optimization of registration and scheduling to improve access to care as well as the development and adoption of self-service tools, the development of a length of stay command center driven by artificial intelligence (“AI”), the expansion of robotics in pharmacy, the establishment of a network operations center powered by AI, employee scheduling using AI, and the maximization of provider time at the bedside by using generative AI for documentation.

NYPH is using digital innovation as a tool to transform how care is delivered, such as using an AI model to detect valvular heart disease and develop an advanced cardiac data mart.

NYPH, the Regional Hospitals, the NYP Medical Groups,¹ Weill Cornell Medicine (the faculty practice of Weill Cornell Medical School) and ColumbiaDoctors (the faculty practice of Columbia VP&S) have all fully implemented Epic, a standardized electronic health record and billing system. Implementation of the Epic Lab Information System was recently launched and is expected to go-live systemwide in the fourth quarter of 2025.

¹ The NYP Medical Groups are a number of medical practices established as New York professional corporations associated with NYPH or one of the Regional Hospitals.

EchoNext: AI-Detection of Structural Heart Disease

Structural heart disease (“SHD”) is a growing epidemic that remains significantly underdiagnosed. The impact is profound, with heart failure and valvular heart disease affecting an estimated 64 million and 75 million people, respectively, with prevalence increasing. Early diagnosis is a clinical priority, yet barriers like cost and limited access to echocardiography (“ECG”) limit its widespread use. Recent advances in AI-based ECG analysis have shown promise but lacked validation in diverse populations across a broad spectrum of SHD.

Under the leadership of Dr. Pierre Elias, researchers at NewYork-Presbyterian/Columbia and Columbia University developed EchoNext, a deep learning model trained on more than 1.2 million electrocardiogram-echocardiogram (ECG-echo) pairs collected from eight of NYPH’s hospitals and affiliated clinics. In a blinded reader study involving 13 board-certified cardiologists, EchoNext demonstrated improved diagnostic performance compared to clinical interpretation alone. These results indicate that advanced diagnostic tools such as EchoNext can enhance physician decision-making by providing additional, high-quality data to support clinical judgment. Importantly, the study does not suggest a replacement of clinical expertise, but rather emphasizes the potential for technology to augment the capabilities of medical professionals. Further supporting its clinical utility, a 100-patient prospective trial found that 73% of patients identified as high-risk by EchoNext were newly diagnosed with structural heart disease (SHD).

These results have been accepted for publication in *Nature* and support future clinical trials, including a large-scale randomized controlled trial aimed at evaluating the impact of AI-based ECG screening on SHD diagnosis.





Investment in Heart Failure Treatment Through Artificial Intelligence (AI) and Machine Learning

NYPH, with physicians from the Medical Schools, is collaborating with Cornell Tech and the Cornell Ann S. Bowers College of Computing and Information Science (“Cornell Bowers CIS”). Cornell Tech is a technology, business, law, and design campus of Cornell University located on Roosevelt Island in Manhattan, New York City. This collaboration is designed to transform cardiovascular health and heart disease prediction and prevention using AI and machine learning. NYPH is supporting Cornell Tech with \$15.0 million over a period of years through 2026 to fund an initiative that aims to improve heart failure treatment, as well as predict and prevent heart failure.

Researchers from Cornell Tech and Cornell Bowers CIS are collaborating with physicians from the Medical Schools to use AI and machine learning to examine multi-modal data and detect patterns to help predict who will develop heart failure, inform care decisions, and tailor treatments for patients. The initiative brings together the technical, clinical, and research expertise in cardiovascular medicine of NYPH and the Medical Schools with Cornell Tech and Cornell Bowers CIS’s leadership in advanced machine learning and AI.

Cybersecurity

Like many other large organizations, NYPH relies on digital technologies to conduct its operations. In the past several years, individuals and entities, often state-funded, have sought to gain unauthorized access to digital systems of large organizations for the purpose of disrupting operations, exfiltrating data, interfering in patient care, and other reasons. These attempts include highly sophisticated efforts to electronically circumvent network security, as well as more traditional intelligence gathering and social engineering aimed at obtaining information necessary to gain access. NYPH maintains sophisticated detection capabilities centering on identity behavior, identity-to-account privilege management, and endpoint detection and response. This is backed up by a highly trained and specialized Security Operations Center and Security Operations Support Team.

While NYPH maintains a network security system designed to stop cyberattacks by third parties and minimize their impact on operations, no assurances can be given that its network security systems will be completely successful. NYPH’s Security Operations Center therefore invests an equal amount of time focusing on both prevention and detection prior to a cyberattack and response, recovery and mitigation of damage for the period following an attack, including system recovery planning and business continuity planning.

New Facilities in New Markets to Serve More Patients

NEWYORK-PRESBYTERIAN OCH SPINE

The NYP Och Spine facility at the Spiral Building in Hudson Yards in New York, New York opened in January 2025. The NewYork-Presbyterian Och Spine (“NYP Och Spine”) is a collaboration between NYPH with ColumbiaDoctors and NYPH with Weill Cornell Medicine that includes many of the world’s top spine specialists (including, orthopedic and neurosurgical spine surgeons, physical medicine and rehab physicians, pain management physicians, and neurologists) who treat children and adults with spine problems from the simple to the complex on an inpatient and outpatient basis. The 75,000-square-foot facility in the Spiral Building is a destination outpatient center for patients seeking treatment at NYP Och Spine. It houses medical practice space, procedure rooms, imaging services, and physical therapy services.

In August 2023, NYPH opened Och Spine at NYP/Weill Cornell. This 18-bed neuro-spine inpatient unit has all private rooms, a dedicated rehabilitation therapy space, and video electroencephalogram (VEEG) epilepsy monitoring capabilities. Och Spine at NYP/Weill Cornell is staffed with a multidisciplinary team to create customized care plans designed to deliver the highest standard of care for neck and back pain.





WESTCHESTER EXPANSION

NYPH's expansion of services in Westchester County continues with the goal of providing residents easier access to top doctors in a wide range of specialties. In July 2023, clinicians with NYP Och Spine began providing patient care at NYP Westchester. Additionally, in September 2025, NYPH expects to open a multispecialty ambulatory care facility at 1111 Westchester Avenue in White Plains. This facility is located adjacent to I-287, close to the Hutchinson River Parkway and I-95 interchanges, providing convenient access for patients throughout Westchester, Bronx, and Rockland Counties in New York, and Fairfield County in Connecticut. The facility will have a Center of Excellence approach, delivering one-stop care for neurosensory and cardiovascular patients, as well as comprehensive services for men's, women's and children's health. This ambulatory care facility has been collaboratively planned with ColumbiaDoctors and will offer imaging services and additional hospital-based outpatient services including cardiac catheterization, electrophysiology and radiation therapy. NYPH has also supported the creation of The NewYork-Presbyterian Iona School of Health Sciences at Iona University's Bronxville campus.

BROOKLYN EXPANSION

In November 2024, NYP Brooklyn Methodist opened a new obstetrics and gynecology practice and imaging center in Park Slope, Brooklyn. This practice and imaging center is located within NYP Brooklyn Methodist's Center for Community Health, a 400,000 square-foot ambulatory center adjacent to its inpatient facility. It offers prenatal testing and imaging, and gynecologic scans and breast imaging, and has 21 exam rooms.

NYPH is expanding its ambulatory footprint with a new 16,500-square-foot, multispecialty site in Bay Ridge, Brooklyn. Strategically situated near the 86th Street subway station, the Belt Parkway, and the Gowanus Expressway, this location is accessible to patients throughout Brooklyn. This expansion is fueled by a significant growth in the ambulatory market in recent years and demonstrates NYPH's commitment to meeting the evolving healthcare needs of the community. The new facility will also enable NYPH to extend care and treatment to the patients in this community where nearly one in four local residents currently seek inpatient care outside of the Borough of Brooklyn. The new site is scheduled to open in early 2026 and has been collaboratively planned with Weill Cornell Medicine to support the needs of the community, offering primary care, medical subspecialties, imaging, and ancillary testing services.



NEW ONCOLOGY AND MULTISPECIALTY BUILDING AT NYP/COLUMBIA

NYPH is investing over \$1.0 billion to build a new state-of-the-art facility at NYP/Columbia, the first major development on this campus since 2010. Located at the site of the former Harkness Eye Institute and connected to NYP Morgan Stanley Children's Hospital, this new center, to be known as The Beacon, will bring together the Herbert Irving Comprehensive Cancer Center, world-renowned pediatric cardiac services, and multispecialty care in a single, patient-centered facility. This facility will total approximately 515,000 gross square feet and will include two inpatient pediatric intensive care units, two inpatient operating rooms, twelve outpatient operating rooms, and four endoscopy suites. It will also include radiation oncology, infusion, imaging, multispecialty clinics, and provide a new home for the Harkness Eye Institute. The consolidation of services at The Beacon also enables NYPH to accelerate its shift to private post-partum rooms and expand its neonatal intensive care unit capacity. The Beacon is a cornerstone of NYPH's vision to further NYP/Columbia as a leading destination for advanced care.

COMMUNITY HEALTH AND BENEFITS

Sustainability

Understanding the strong relationship between health and the environment, NYPH aims to create a healthier environment for patients, staff, and everyone in the communities it serves by adopting sustainable practices. Through innovations in energy strategy, building design, procurement policies, and clinical practice, NYPH seeks to embed sustainability into its operations.

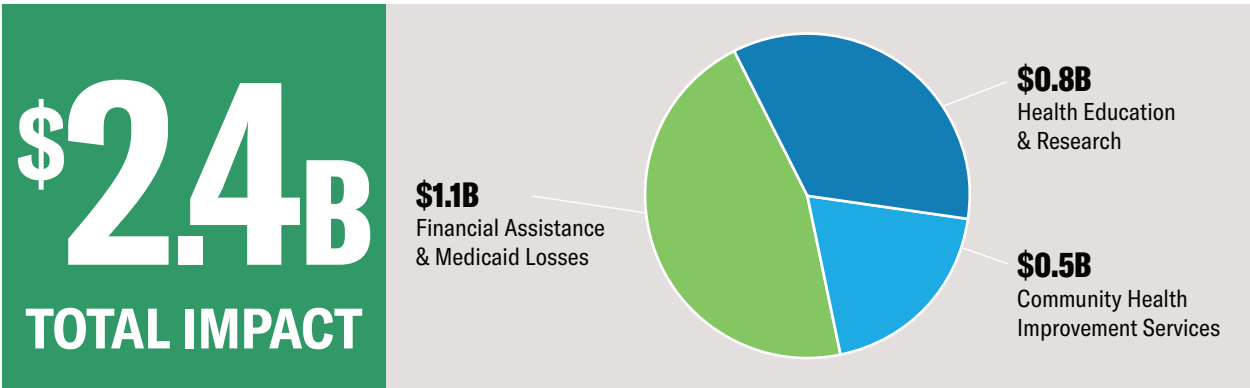
- NYPH continues to invest in energy efficiency infrastructure projects, which include steam trap replacements, control upgrades, heat recovery from exhaust air, energy audits, air handler replacements, pipe insulation improvements, and LED lighting upgrades. These efforts help to reduce energy consumption while improving overall building efficiency.
- In addition to reducing energy intensity, NYPH is sourcing renewable electricity through power allocations from New York Power Authority's "ReCharge NY" program. In 2024, more than 10% of hospital-campus electricity for NYPH and the Regional Hospitals was supplied from renewable sources.
- A 600kW Solar PV Array located on the parking garage at NYPH's 1111 Westchester Avenue location is now live. This is the first solar array installed at any NYPH facility. It is expected to generate more than 5 million kilowatt hours of low-carbon electricity for this location.
- NYPH also seeks to achieve Leadership in Energy and Environmental Design ("LEED") certification from the U.S. Green Building Council ("USGBC") on new construction projects. To date, more than 1.8 million square feet of space at NYPH locations has LEED designation.
- In 2024, three NYPH campuses were recognized by the American Society for Health Care Engineering ("ASHE") with "Energy to Care" awards for sustained reductions of energy consumption: NYP/Columbia, NYP Allen, and NYP Westchester Behavioral Health.
- Working with The Afya Foundation, a supply rescue organization, NYPH and the Regional Hospitals have diverted to humanitarian efforts in the community and across the globe nearly 550,000 pounds of medical supplies and equipment they no longer need.



Advancing the Health and Well-Being of Communities Through Benefits, Programs, and Partnerships

COMMUNITY BENEFITS

NYPH and the Regional Hospitals are dedicated to the health and well-being of the communities they serve. For the year ending December 31, 2023, NYPH contributed \$2.4 billion to community benefits programs (as defined by the Internal Revenue Service), which includes financial assistance, Medicaid shortfalls, subsidized health services, community health improvements, and health professions education and research. The Medicaid expansion under the Affordable Care Act led to higher Medicaid shortfalls, significantly impacting hospitals like NYPH that provide extensive care to vulnerable populations. NYPH believes that supporting these essential programs provides an important community benefit. Consequently, NYPH and the Regional Hospitals allocated \$1.1 billion for financial assistance and Medicaid losses, \$0.8 billion for health education and research, and \$0.5 billion for community health improvement services.



As part of its effort to provide compassionate, high-quality healthcare to all, regardless of socio-economic status or ability to pay, NYPH assesses individual patient needs comprehensively and offers financial aid to eligible patients, irrespective of age, gender, race, national origin, socio-economic status, sexual orientation, or religious affiliation. NYPH has an established Charity Care program which assists patients facing financial hardship, offering financial assistance to those with household incomes ranging from 0% to 600% of the Federal Poverty Guidelines.

NYPH is committed to supporting the critical work of community-based organizations (“CBOs”) in improving the health and wellbeing in the neighborhoods it serves. This includes capacity-building work for local nonprofits that includes grants, as well as technical assistance to help nonprofits grow and thrive, sessions on quantitative and qualitative evaluation, and local affinity group meetings reaching more than 60 organizations. NYPH held its annual Capacity Building Conference on November 19, 2024, which was attended by more than 100 CBOs.

In addition, NYPH supports CBOs with a range of funding opportunities to help them address social determinants of health. Over the six months ended March 31, 2025, NYPH distributed \$3.5 million in grant funding to support a range of CBO-led activities, including activities related to employment, housing and other quality of life issues. Grants provided \$600,000 to support “Breaking Ground”, an initiative to

address homelessness around two NYPH campuses, \$200,000 to support the New York City Employment & Training Coalition to increase the number of employees NYPH hires from local communities, and over \$2.0 million to grantees who are addressing social determinants of health across NYPH's service area. An additional \$700,000 will support grant awards in 2025 for new and returning grant recipients under NYPH's mini-grant program and its newly launched Summer Youth Enrichment Grants.

COMMUNITY PROGRAMS

NYPH's Ambulatory Care Network provides clinical services at primary care and specialty practice sites, school-based health centers and mental health programs, serving generations of families across New York City representing a wide array of nationalities and ethnic and religious backgrounds, with a goal of making it easier for everyone to access high quality, affordable, patient-friendly care in their own neighborhoods.

NYPH has extensive community health programming and a long history of working with community organizations to address the root causes of disease and reduce health inequities. In January 2024, NYPH became one of the first hospitals in the nation to be awarded a Health Care Equity Certification by The Joint Commission for its work to address and improve health care equity across the healthcare system. Caring for all New Yorkers is essential to NYPH's mission and a key part of its strategy.



NYPH's community-based youth programs continue to grow and receive external recognition. In 2024, NYPH's Uptown Hub, which provides a safe space as well as behavioral health services to hundreds of youth between the ages of 18 and 24, was selected as the Honoree for the 2024 New York State Office of Mental Health's "What's Great in Our State" Award.

NYPH recognizes there can be barriers in accessing care, particularly for vulnerable New Yorkers. In 2024, to better reach people where they are, NYPH launched two advanced technology mobile medical units: a Sexual Health Mobile Medical Unit (the "SH Mobile Unit") and a Mom and Baby Mobile Medical Center. The SH Mobile Unit offers comprehensive sexual health services for all, with a focus on addressing HIV and sexual health disparities in Brooklyn, Queens, Harlem and Washington Heights. The Mom and Baby Mobile Medical Center, launched in collaboration with the March of Dimes, provides free, high-quality health care to women who are pregnant or of childbearing age. To promote health and disease prevention outside of brick-and-mortar institutions, NYPH has also worked with community organizations to create pop-up clinics that provide benefits of early detection and intervention. Through March 31, 2025, as part



of these initiatives, NYPH has collaborated with 17 community organizations across New York City and provided services to over 460 patients.

To better support its patients, NYPH established a clinic for patients diagnosed with a substance use disorder called TRANSiT (Therapeutic Resources and Assistance in Navigation to Services while in Transition). This initiative was driven by the high volume of patients—over 2,600 individuals—who accessed the emergency department and inpatient services with substance use concerns over an 18-month period. Recognizing the need for immediate, low-barrier access to care, the clinic provides timely support and comprehensive treatment options to bridge the gap between emergency visits and long-term recovery. From May 1, 2024 through March 31, 2025, this clinic had a total of 806 visits.

In September 2024, NYPH introduced its Health Advocacy campaign aimed at encouraging New Yorkers to adopt simple, daily behavioral changes that promote overall health and well-being. The campaign provides practical tips, such as engaging in regular exercise, maintaining a balanced diet, and prioritizing sufficient sleep. These health suggestions are shared through a series of digital advertisements and citywide displays with messaging designed to engage the community. To further promote heart health, NYPH installed live speedometers at select bus stops in Brooklyn, Queens, and Manhattan, encouraging pedestrians to increase their walking pace. These initiatives align with NYPH's commitment to improving public health outcomes.

The current campaign is set to run through the end of the year, with plans to introduce new health tips and insights in the coming year.

Nutrition and physical activity remain a cornerstone of NYPH's community and population health work. Since March 2020, NYPH's food insecurity program, Food FARMacy, has teamed up with more than twelve community-based organizations to provide food to patients and families in need in local communities. From January 1, 2023 through March 31, 2025, NYPH's home delivery program has distributed over 2.0 million pounds of healthy food valued at over \$5.5 million to over 4,000 unique households, representing over 14,700 individuals. NYPH's Youth Market program provides a paid internship for young people living in neighborhoods heavily affected by food insecurity. One hundred and three interns have graduated since the program launched in 2019. NYPH's Health 4 Life program continues to serve pediatric patients who are

overweight or obese through an interdisciplinary care model. The program is expanding to NYP Queens in 2025. NYPH's My Block program aims to increase access to safe places to play, move, and connect across New York City and Westchester County. The program has collaborated with twelve community-based organizations and reached 5,500 community members since launching in January 2024.

In September 2024, NYPH was awarded a Solutions to End Homelessness Program grant by the New York State Office for Temporary and Disability Assistance to address housing insecurity faced by NYPH patients. With this grant, which provides \$300,000 per year over three years, NYPH is expanding its collaboration with Breaking Ground, a community-based organization focused on creating high-quality permanent and transitional housing for the homeless, moving beyond the Washington Heights area to lower Manhattan. Together with Breaking Ground, navigators will be stationed in the NYP Lower Manhattan emergency department to provide patients with comprehensive, personalized support and connect them to long-term housing solutions and services that prioritize health and stability.

The Dalio Center for Health Justice

Now in its fourth year, the Dalio Center for Health Justice (the “Dalio Center”) was established with the foundational support of a \$50.0 million grant from Dalio Philanthropies, whose founder Ray Dalio is a member of the NYPH Board of Trustees. The Dalio Center’s work is made possible by this original grant, with additional support from Lisa & Mark Schwartz and the Samuel I. Newhouse Foundation.

The Dalio Center aims to understand and address the root causes of health inequities. Together with NYPH and people and organizations in its surrounding communities, the Dalio Center invests in research and education, advocates for policy change, and implements clinical and community strategies that address health inequities.





Human Capital

NYPH is dedicated to creating a healthier future for all and encourages its staff to provide the highest quality, most compassionate care to the patients and communities it serves. In June 2024, NYPH launched a “Values and Actions” campaign, which focuses on delivering excellence through respect, integrity, empathy, teamwork, and innovation.

NYPH recognizes that employee wellbeing is paramount to the delivery of quality care to patients and the community. As such, NYPH maintains a robust health and wellbeing program that focuses on various domains of wellbeing, including emotional resilience, physical health, work-life harmony, financial stability, career mobility and community stewardship. NYPH offers programming in all these areas, and its onsite wellbeing coaches help employees navigate resources in order to achieve personal wellbeing goals.

NYPH offers the CopeNYP Employee Assistance Program (“CopeNYP”) to support NYPH employees and their families. Staffed by licensed mental health professionals from Weill Cornell Medicine’s Department of Psychiatry, CopeNYP can provide emotional support and practical strategies to enhance the client’s resilience. CopeNYP can address a wide variety of concerns, such as managing personal and work stressors (including burnout), reducing anxiety, alleviating depression, addressing relationship issues and interpersonal conflicts (among couples, family or friends), coping with traumatic events and grief and loss, as well as helping teens cope with their issues.

In recognition of NYPH’s commitment to support employee mental health and overall wellbeing, Mental Health America awarded NYPH the 2024 Platinum Bell Seal for Workplace Mental Health, the highest award level offered.

With a significant contribution from a member of the NYPH Board of Trustees, NYPH operates the “NYP Cares for Employees Fund.” This fund helps to support employees who are facing unexpected crises or significant hardships. The NYP Cares for Employees Fund includes two programs: an employee hardship grant program and an employee food bank. The first of these, the NYP Employee Hardship Grant program, provides grants of up to \$5,000 to eligible employees facing unanticipated, significant financial hardship due to a qualifying emergency, such as a natural disaster, residential fire, housing instability, death of a household member, military deployment, or domestic abuse. The second, the

NYP Employee Food Bank, provides eligible employees and their families experiencing food insecurity with temporary food assistance delivered directly to their home. NYPH will provide one month of food staples to bridge those in need as they transition to a longer-term solution.

In 2024, NYPH enhanced its financial support offerings by collaborating with GreenPath Financial Wellness. GreenPath offers NYPH employees free financial counseling and can provide budgeting support, credit report consultation, and debt management planning.

Culture: Delivering Excellence Through Shared Values and Actions

NYPH and the Regional Hospitals seek to foster a culture of belonging, inclusion, and respect. By highlighting the culture that enriches the NYPH and Regional Hospital community, NYPH and the Regional Hospitals seek to deepen an understanding of the traditions, experiences, and perspectives of others. Through the “Dialogues” speaker series, team members explore issues of inclusion and belonging. Inclusion Groups offer a safe space for all employees to continue dialogue about inclusion and belonging through a lens of intersectionality and togetherness. Inclusion Groups operate at every NYPH and Regional Hospital campus and are open to all.

To support community members facing food insecurity, NYPH and Regional Hospital employees participate in food drive efforts, working with local community-based organizations to donate food to those in need. This includes collaborations with the NY Common Pantry, CHiPS Soup Kitchen and Shelter, Caring for the Hungry and Homeless of Peekskill, Hope Community Services, the Loaisaida Community Center, the Mexican Coalition, Nido de Esperanza and the Thomas H. Slater Center,

NYPH strives to have a supplier base that reflects and supports the patient population and communities it serves. Through the NewYork-Presbyterian Supplier program, NYPH seeks to engage small and community-based businesses both locally and throughout the country.

NYPH’s efforts have led to its recognition as A Great Place to Work by Great Place to Work®, a global company focused on workplace culture.



NYPH AND REGIONAL HOSPITALS AWARDS AND RECOGNITIONS



The Joint Commission

NYPH, NYP Queens, and NYP Hudson Valley each hold **The Joint Commission's Gold Seal of Approval Hospital Accreditation and Behavioral Health Care & Human Services Accreditation.**

In January 2024, NYPH became one of the first hospitals in the nation to be awarded a **Health Care Equity Certification** by **The Joint Commission** for its work to address and improve health care equity across the healthcare system. To receive the certification, NYPH was required to produce comprehensive reports and data on its efforts to address health disparities. The rigorous review by The Joint Commission looked at over 40 program-specific standards, including patient experience, quality and safety measures, staff training, recruitment and retention, and community partnerships. NYP Queens and NYP Hudson Valley anticipate achieving The Joint Commission's Health Equity certification in the near future.

The following NYPH campuses, Regional Hospitals, and programs hold the following certifications from The Joint Commission:

- Ventricular Assistance Device (NYP/Columbia and NYP/Weill Cornell bi-campus certification)
- Comprehensive Stroke Center (NYP/Columbia and NYP/Weill Cornell bi-campus certification)
- Thrombectomy Capable Stroke (NYP Brooklyn Methodist and NYP Queens)
- Primary Stroke (NYP Allen, NYP Lower Manhattan, NYP Westchester, and NYP Hudson Valley)
- Orthopedics-Hip and Knee (NYP Queens)



Centers for Medicare & Medicaid Services (CMS)

In 2024, for the second year, CMS awarded NYPH a Five-Star rating, CMS's highest rating, in its annual quality measurement program for hospitals. Ratings for 2025 have not yet been released. The rating was driven by improvements in key patient safety indicators, including mortality, patient experience scores, and hospital-acquired infection rates that were achieved by a focus on high-reliability and evidence-based practice.

CMS Star Rating, which CMS developed in collaboration with key stakeholders to calculate overall hospital-level quality, provides consumers with an overall rating generated by combining multiple dimensions of quality into a single summary score.

U.S. News & World Report's Best Hospitals and Best Children's Hospitals Rankings 2024-2025

In July 2024, NYPH was again recognized as one of the best hospitals in the nation and a top hospital in New York by **U.S. News & World Report**. This is the 21st year in a row that NYPH has been named among the top hospitals in the nation. In the 2024-2025 Best Hospital's Report, NYPH ranks in the top five nationwide in seven specialties: cardiology, heart & vascular surgery (No. 5); neurology and neurosurgery (No. 4); orthopedics (No. 4); psychiatry (No. 3); diabetes and endocrinology (No. 2); urology (No. 2); and rheumatology (No. 3), a collaborative program with Hospital for Special Surgery. NYPH ranked in the top 20 in 14 of 15 specialties, more than any other hospital in New York.

NYPH is one of just 20 hospitals nationwide to be named to the 2024-2025 Best Hospitals Honor Roll, a distinction recognizing hospitals that deliver the highest quality of care across a range of specialties, procedures, and conditions.

For the 2024-2025 rankings and ratings, *U.S. News* evaluated nearly 5,000 hospitals across 15 specialties and 20 procedures and conditions. Only 11% of hospitals evaluated earned a Best Hospitals ranking, which is based on clinical outcomes, level of nursing care, and patient experience, among other factors.

In October 2024, NYPH was recognized as the #1 children's hospital in New York, by **U.S. News & World Report's** 2024-2025 "Best Children's Hospitals" rankings. This is the 18th year in a row that NYPH (encompassing pediatric care in every area of medicine at two major sites: NYP Morgan Stanley and NYP Komansky Children's Hospital at NYP/Weill Cornell) has ranked among the best children's hospitals in the nation. NYPH was the only hospital in New York to rank among the top 50 hospitals across all 11 pediatrics specialties evaluated. NYPH ranked in the top 20 in five pediatric specialty areas: cardiology & heart surgery, diabetes & endocrinology, gastroenterology & gastrointestinal surgery, neurology & neurosurgery, and pulmonology. NYPH was also named one of the nation's best in pediatric and adolescent behavioral health, a new *U.S. News* ranking this year.

NYPH was also included on the Newsweek/Statista's list of **America's Best Hospitals 2024** as a Best Cardiac, Cancer, Neurological, Orthopedic and Children's Hospital.

Quality & Safety

In September 2024, Vizient, one of the nation's leading healthcare performance improvement companies, recognized NYPH with two prestigious awards at its annual healthcare summit. **NYP/Weill Cornell** was honored as a **Vizient Top Performer Rising Star**, for significant improvement in care quality by an academic medical center. This award is measured by a rise in at least 20 positions in the annual Vizient Quality and Accountability Study from the prior year and scoring in the top 25. **NYP Brooklyn Methodist** was honored with **Vizient's Top Performer-Bernard A. Birnbaum, MD, Quality Leadership Award** for scoring in the top ten within its Complex Care Medical Center cohort. The Birnbaum award recognizes performance on the quality of patient care in six domains: safety, mortality, effectiveness, efficiency, patient centeredness, and equity. The study factors in data from the Hospital Consumer Assessment of Healthcare Providers and Systems survey and the Center for Disease Control's National Healthcare Safety Network.

All other NYPH campuses scored in the top third of their respective categories, making a significant year-over-year improvement.

Team Member Engagement



In March 2025, NYPH was recognized with a 2025 Gold Well Workplace Award and listed as one of America's Healthiest Companies by the **Wellness Alliance**, a leading professional association dedicated to wellness in individuals and organizations across the United States. Gold Well Workplace Awards honor organizations that have built exceptional and comprehensive workplace wellness strategies grounded in a number of key benchmarks, such as Committed and Aligned Leadership, Collecting Meaningful Data, Cultivating Supportive Health Promoting Environments, and Policies and Practices.



In January 2025, **Forbes** included NYPH on its list of **America's Best Employers for 2025**. The ranking is based on independent survey responses from over 217,000 U.S. employees working at companies with a national workforce of at least 1,000 people. The survey evaluates employees' personal and public assessments on key dimensions, including atmosphere & development, salary & wage, company image, working conditions, and workplace environment.

In December 2024 **Computerworld**, a leading IT Industry publication and online resource included NYPH on its list of the **Best Places to Work in IT in 2025**. The publication surveyed large, midsize, and small organizations to find the top IT employers. Criteria included salary and benefits as well as working on challenging projects, growth opportunities, and continuous learning in a supportive workplace.



In November 2024, **Forbes** placed NYPH at #19 on its inaugural list of **America's Dream Employers for 2025**. Forbes partnered with market research firm Statista to survey more than 10,000 college students in the United States and more than 140,000 U.S.-based employees at companies across all sectors that employ at least 1,000 people in the country. The students were asked which employers they considered to be dream employers and to rate how excited they would be to receive an offer from the company.

In October 2024, NYPH's IT Department was recognized with a **Webex Customer Award** and designation as a **Bold Explorer in Healthcare**, for NYPH's success in integrating Webex Connect with the EPIC Electronic Health Record as a means for streamlining virtual rounding for clinical teams. The new workflow has improved rounding times and electronic health record logging for documentation and billing.



In September 2024, **Forbes** named NYPH to its list of **Best-in-State New York Employers** for 2024. The business media company partnered with market research firm Statista to survey 160,000 people working in companies with at least 500 employees within the United States. In maintaining a workplace that supports growth, NYPH continues to rank in the top 20 Best-in-State Employers in New York for two years. Focusing on employee wellbeing and advancement opportunities, NYPH provides career training, tuition reimbursement, leadership preparation, and a variety of wellness programming for team members.

In August 2024, **Aetna** recognized NYPH with its highest-level **Making a Difference, Workplace Wellbeing Platinum Award**, for maintaining and enhancing a quality workplace environment by continually integrating effective programming, tools, and resources tailored to each individual's well-being. Highlights in 2024 include wellbeing coaches available at all campuses, the CopeNYP Employee Assistance Program staffed by licensed mental health professionals from Weill Cornell Medicine's Department of Psychiatry, providing short-term counseling sessions, free of charge, and the NYP Cares for Employees Fund offering critical assistance to team members facing unexpected crises and emergencies.

In July 2024, NYPH was recertified as a Great Place to Work for 2024 by **Great Places To Work**, a global authority on workplace culture, employee experience and leadership that conducts annual surveys based on rigorous methodology and validated employee feedback. NYPH's intensive focus on building and sustaining an organizational culture rooted in respect, equity, inclusion, belonging, teamwork, employee wellness and professional development has contributed to NYPH's strong showings on Great Places to Work Surveys since 2017. Results of the 2024 survey show that 81% of employees at NYPH say it is a great place to work, up from 80% in 2023.



In July 2024, **Forbes** named NYPH on its list of **Best Employers for Women**. Partnering with the market research firm Statista, Forbes surveyed approximately 150,000 women working at companies based in the US with at least 1,000 employees. The survey asked participants to rate their current employers, considering factors involving the work environment, pay equity, parental leave, and the company's handling of incidents related to misconduct and discrimination.



In May 2024, **Forbes** included NYPH on its list of **Best Employers for New Graduates**, recognizing NYPH as an industry leader committed to nurturing and supporting the development of young professionals. Forbes/Statista surveyed more than 100,000 young professionals working for companies with at least 1,000 employees in the United States. Respondents were asked whether they would recommend their employer to others, and to rate that organization in such areas as compensation, benefits, advancement opportunities, working conditions, flexibility, company image, and inclusion.

In May 2024, the **Dave Thomas Foundation for Adoption** placed NYPH as number 8 on its list of the 100 Best Adoption-Friendly Workplaces in the United States. Participants were scored based on three criteria: financial reimbursement, paid leave, and the percentage of employees eligible for those benefits.



In May 2024, the **National Academy of Medicine's Change Maker Campaign for Healthcare Workforce Wellbeing** designated NYPH a Change Maker, i.e., an organization that is driving collective action to turn the tide of health worker burnout, rebuild a thriving health profession, and strengthen healthcare workforce wellbeing. NYPH wellbeing offerings include easily accessible programs such as financial education, mental health counseling, recognition programs, gratitude practices, social support, emotional wellbeing, physical health, career guidance, and much more.



In April 2024, NYPH was awarded the **Platinum Bell Seal for Workplace Mental Health** by **Mental Health America**—the highest award level offered. The Bell Seal is a first-of-its-kind workplace mental health certification that recognizes employers who strive to create mentally healthy workplaces for their employees. Factors contributing to NYPH's success this year include comprehensive employee wellbeing offerings, commitment to a culture of equity, inclusion, and belonging, and improved and streamlined access to employees seeking out mental health support through the CopeNYP/CopeGME program.

Excellence in Nursing



The **American Nurses Credentialing Center Magnet Recognition Program®**, the highest and most prestigious distinction that a healthcare organization can earn for nursing excellence and innovation, has designated Magnet status on the following NYPH and Regional Hospital campuses and programs (the year of initial designation is indicated in parenthesis):

- NYP Allen Hospital and NYP Ambulatory Care Network West (2021)
- NYP Morgan Stanley (2021)
- NYP Lower Manhattan (2020, submitted for redesignation in October 2024)
- NYP/Weill Cornell (2019, redesignated in 2024)
- NYP/Columbia (2019, redesignated in 2024)
- NYP Westchester Behavioral Health and Weill Cornell Medical Center Psychiatry Program (2018, redesignated in 2024)
- NYP Hudson Valley (2007, redesignated in 2021)

Applications for Magnet designation for NYP Westchester, NYP Brooklyn Methodist and NYP Queens have been submitted. A Magnet site visit is scheduled on May 28-30, 2025 for NYP Westchester and on June 25-27, 2025 for NYP Queens. Document submission occurred in February 2025 for NYP Brooklyn Methodist. Gracie Square Hospital, affiliated with NYPH, received initial designation in 2022.

Recruitment and Retention

In November 2024, NYPH received a **2024 Global Candidate Experience Award** as part of the **ERE Media and the CandE Benchmark Research Program**. 125 companies hiring worldwide registered to participate in the program, which ultimately collected the thoughts and experiences of over 230,000 job candidates. The largest study of its kind, ERE Media evaluated responses from over 2,000 global employers and over 1.8 million job seekers in recognizing NYPH and other organizations that offer a positive experience to job candidates.

Environmental Sustainability

NYPH recognizes the intrinsic relationship between the health of people and the health of the environment. By adopting sustainable practices in the hospital and community, NYPH aims to create a healthier environment for patients, staff, and the communities it serves.



- In June 2024, the **American Society for Health Care Engineering (“ASHE”)** recognized NYPH for its leadership in setting the standard for energy stewardship in health care. Energy saving initiatives at NYP/Columbia, NYP Allen, and NYP Westchester earned these facilities ASHE recognition as Sustained Performance Award winners.
- NYPH also seeks to achieve **Leadership in Energy and Environmental (“LEED”)** certification from the **U.S. Green Building Council (“USGBC”)** on new construction projects. In 2023, NYP Brooklyn Methodist’s Center for Community Health received LEED Silver Certification® under USGBC’s Healthcare rating system.
- Also in 2023, NYPH received the **Vizient Environmentally Preferred Sourcing Excellence Award**, which recognized NYPH for having the highest percentage of spend on environmentally preferred products across all Vizient members in the Comprehensive Academic Medical Center cohort.

Leaders Recognized for Contributions to Health Care Across the Region and Nation



- In April 2025, Devika Mathrani, Senior Vice President and Chief Marketing Officer at NYPH was named a **2025 Forbes Entrepreneurial CMO 50**. She and 49 other leaders from around the world were recognized for actions and impact that are helping transform their industries and the culture at large.
- In March 2025, **Becker’s Hospital Review** recognized Steven J. Corwin, MD, President and Chief Executive Officer at NYPH on its list of **Great Leaders in Healthcare 2025**.



- In February 2025, **Becker's Hospital Review** listed Deepa Kumaraiah, MD, MBA, Group Senior Vice President, Chief Medical Officer, and Chief Operating Officer, Perioperative Services at NYPH, among the nation's **180+ Chief Medical Officers to Know 2025**.
- In February 2025, **Becker's Hospital Review** included Shaun E. Smith, Senior Vice President, Chief People and Culture Officer at NYPH, on its list of **99 Hospital and Health System CHROs and Chief People Officers to Know 2025**.
- In December 2024, **Beckers' Hospital Review** named Brian G. Donley, MD, Executive Vice President and Chief Operating Officer at NYPH, to its **118 COOs to Know 2024** feature.
- In June 2024, Anand Joshi, MD, MBA, NYPH's Senior Vice President of Procurement and Strategic Sourcing, was honored for demonstrating excellence in healthcare supply chain performance and leadership by the **Bellwether League Foundation** ("BLF")—a national nonprofit organization dedicated to recognizing professionals for their contributions to the healthcare supply chain and in turn, healthcare delivery. He was inducted to the BLF's **Supply Chain Hall of Fame** as a **Dr. Dean S. Ammer Honoree**, which recognizes executives and professionals in the middle of their careers and serves as the bridge between the "Future Famers" at or near the beginning of their careers and the "Bellwethers" who are honored for their entire career span.
- In July 2024, Wilhelmina Manzano, Group Senior Vice President and Chief Nurse Executive at NYPH was honored by the **Daisy Foundation and Health Impact** with a prestigious **Daisy Nurse Leader/Policy Award**. Selected by a national panel of healthcare policy experts, the award honorees are nurses whose policy-oriented and advocacy work improves population health and advances compassionate care.
- In June 2024, **City & State Media** recognized Juan Mejia's leadership as President of NYP Brooklyn Methodist by listing him in its **Brooklyn Power 100** feature.
- In June 2024, **Becker's Hospital Review** included Michael P. Breslin, NYPH's Group Senior Vice President, Chief Financial Officer & Treasurer, among its national list of **160 CFOs to Know 2024**, noting he is responsible for financial planning, reporting, budgeting and revenue cycle, as well as for overseeing the health system's reimbursement strategy, cost accounting, insurance, financing, managed care and supply chain.
- In April 2024, Anne Sperling, Vice President for Governmental Relations & Community Affairs, was recognized by **City & State Media's Who's Who in Government Relations** feature as a notable New Yorker advocating for change in city, state, and federal government. Late last year Ms. Sperling was also recognized by **City & State** for her contributions to social responsibility in New York.
- In February 2024, Patrice S. Dupart, PharmD, MSHCM, BCPS, Vice President of Pharmacy and Apothecary-In-Chief was included in **Becker's Hospital Review's 75 Hospital and Health System Chief Pharmacy Officers to Know 2024**.

Other Notable Recognitions

Many awards and recognitions earned by NYPH and the Regional Hospitals in 2024 and 2025 demonstrate a commitment to clinical excellence, quality patient care, and safety.

NYP/Columbia, NYP/Weill Cornell, NYP Brooklyn Methodist, NYP Westchester and NYP Queens received A ratings in the Leapfrog Hospital Safety survey for Fall 2024, reflecting significant structural improvements around patient safety and healthcare acquired infection scores.

In December 2024, NYPH's Prevention and Education in Advanced Kidney Disease (PEAK) program was named a top finalist for the Bernard J. Tyson National Award for Excellence in Pursuit of Healthcare Equity. The Tyson award is an annual recognition of health care organization initiatives that achieve measurable, sustained reductions in health care disparities. The PEAK program is a collaborative effort of the Dalio Center and the Rogosin Institute, an affiliate of NYPH, focused on addressing disparities in end-stage kidney disease. It provides comprehensive support, including education, medical expertise, and social support, to patients at NYP/Columbia and NYP Allen.

In October 2024, The American College of Surgeons National Surgical Quality Improvement Program ("ACS NSQIP") recognized NYP Brooklyn Methodist, NYP/Weill Cornell, and NYP Queens as among 77 of 609 eligible hospitals participating in the ACS NSQIP adult program that achieved meritorious outcomes for surgical patient care. ACS NSQIP-Meritorious hospitals were commended for using data effectively in achieving optimal patient outcomes.

In October 2024, the **American Association for Respiratory Care** named NYP Brooklyn Methodist's Respiratory Therapy Department a 2024 recipient of its Apex Recognition Award. NYP Brooklyn Methodist is one of only 29 hospitals in the nation to receive this recognition. The Apex Recognition Award is awarded to hospitals demonstrating best advanced practices, the promotion of patient safety professional development, evidence-based care, patient satisfaction, and quality improvement. This was the sixth year in a row that NYP Brooklyn Methodist has received this recognition.

In September 2024, NYP/Weill Cornell and Weill Cornell Medicine received the **Emergency Medicine Wellness Center of Excellence Award** from the **American College of Emergency Physicians** for commitment to the personal wellness and resilience of its emergency medicine department staff.

The American Association of Critical Care Nurses (“AACCN”) Beacon Award recognizes standards of excellence in patient care environments, and highlights caregivers in units whose consistent and systematic approach to evidence-based care and unit and hospital culture optimizes outcomes. In 2024, AACCN Beacon Award winners include the following NYPH units:

Beacon Award Gold

- NYP/Columbia 5 Hudson North, valid through March 2026
- NYP/Columbia 5GN Cardiothoracic Stepdown, valid through April 2027
- NYP Morgan Stanley 11 Central PICU, valid through April 2027
- NYP/Weill Cornell G3PACU, valid through July 2027

Beacon Award Silver

- NYP/Columbia Peri Anesthesia Care Unit (PACU), valid through November 2026
- NYPH Interventional Radiology/Neuro Radiology, valid through August 2027
- NYP Lower Manhattan 3C-ICU, valid through September 2027
- NYP Morgan Stanley 9 Central PICU, valid through September 2027
- NYP Westchester Cardiac Catheterization Lab, valid through April 2027
- NYP Westchester 3 South ICU, valid through February 2027
- NYP/Weill Cornell 4 South ICU, valid through October 2027

Beacon Award Bronze

- NYP/Columbia 8 Hudson South, valid through September 2027
- NYP Neuroscience Intensive Care Unit, valid through November 2026
- NYP/Weill Cornell Surgical/Trauma ICU—8 South, valid through October 2027

In February 2024, the American Heart Association and the Mitral Foundation recognized NYP/Weill Cornell as a **Mitral Valve Repair Reference Center** for its contribution to advancing best practices in the surgical treatment of mitral valve disease, a common valvular problem that occurs when irregularities develop in the heart valve between the left heart chambers. The Mitral Valve Repair Reference Center Award was developed to identify, recognize, and promote the nation’s recognized medical centers for mitral valve repair surgery. NYP/Weill Cornell earned the award for demonstrating adherence to evidence-based treatment guidelines as well as an ongoing commitment to reporting and measuring quality and outcome metrics specific to mitral valve repairs.

Financial and Operating Information

Utilization

A summary of historical utilization data for the three months ended March 31, 2025 and 2024 for NYPH is presented in the following table. The table is for NYPH only and excludes data regarding the Consolidated Entities.

HISTORICAL UTILIZATION OF NYPH

	Quarters Ended	
	3/31/2025	3/31/2024
Certified Beds (at end of period)	3,402	3,402
Staffed Beds (at end of period) ¹	3,130	3,157
Discharges ²	35,161	36,086
Patient Days ²	253,634	248,599
Staffed Bed Days Available ¹	281,700	287,287
Average Length of Stay (days) ³	6.67	6.61
Case Mix Index – Medicare	2.21	2.21
Case Mix Index – Hospital wide ⁴	1.80	1.82
Average Occupancy (%) ^{1, 5}	90.0%	86.5%
Emergency Room Visits ⁶	102,366	99,551
Outpatient Clinic Visits	195,196	179,635
Ambulatory Surgery Procedures	36,995	35,346
Mental Health Clinic Visits	30,591	28,864

Source: NYPH records

1. Staffed beds reported is based on the average of the number of staffed beds as of the last day of each month in the period. Staffed beds for the quarter ended March 31, 2024 are restated to align with internal statistics adopted beginning in the quarter ended September 30, 2024.
2. Excludes newborns.
3. Excludes psychiatry, rehabilitation, normal newborn, uncoded and length of stay greater than 300 days.
4. Hospital wide CMI is calculated using the payor specific cost weight (All Payor Refined (APR) or Medicare Severity Diagnosis Related Groups (MSDRG)).
5. Occupancy percentages based on staffed bed days available.
6. Includes only patients seen in the emergency room and not admitted.

Sources of Patient Service Revenue

The majority of patient service revenue received by NYPH is derived from programs that are either insured or administered by third-party entities. The following table reports the percentage of net patient service revenue by payor source for the three months ended March 31, 2025 and 2024. The data in the table is for NYPH only and excludes data regarding the Consolidated Entities.

Percent of NYPH's Net Patient Service Revenue by Payor Source

	Quarters Ended	
	3/31/2025	3/31/2024
Payor		
Medicare ¹	25.1%	24.6%
Medicaid ¹	15.1%	14.9%
Commercial carriers and health maintenance organizations	58.5%	59.5%
Self pay & other	1.3%	1.0%
Total	100%	100%

Source: NYPH records

1. Medicare includes Medicare Advantage and Medicaid includes Medicaid Managed Care.

All revenue, statistics and reimbursement information in this Disclosure Report represent historical data and may not be indicative of future activity. NYPH cannot assess or predict the ultimate effect on its operations which may result from existing or future reimbursement legislation or regulations or other changes in law or market conditions.

NYPH has established estimates, based on information presently available, of amounts due to or from Medicare and non-Medicare payors for adjustments to current and prior years' payment rates, based on industry-wide and NYPH-specific data. The current Medicaid, Medicare and other third-party payor programs are based upon extremely complex laws and regulations that are subject to interpretation. As a result, there is at least a reasonable possibility that recorded estimates will change by a material amount when open years are settled, audits are completed, and additional information is obtained. Additionally, non-compliance with such laws and regulations could result in fines, penalties and exclusion from such programs. NYPH is not aware of any allegations of non-compliance that could have a material adverse effect on its financial statements and believes that it is in compliance in all material respects with all applicable laws and regulations.

Summary Statements of Operations and Financial Position

Summary statements of operations and financial position for NYPH are set forth herein. These summary statements reflect financial results for NYPH only. The unaudited information in the summary statements of operations for the three months ended March 31, 2025 and March 31, 2024 and the summary statements of financial position at March 31, 2025 are derived from the Hospital's¹ unaudited consolidated financial statements and supplementary information as of and for the three months ended March 31, 2025 (the "Three Months Ended March 31, 2025 Unaudited Financial Statements"). The summary statements of financial position at December 31, 2024 are derived from the Hospital's audited consolidated financial statements and supplementary information as of and for the years ended December 31, 2024 and 2023 (the "2024 Audited Financial Statements"). The 2024 Audited Financial Statements and the Three Months Ended March 31, 2025 Unaudited Financial Statements include the results of the Consolidated Entities. These consolidated financial statements also contain supplementary information setting forth consolidating statements of financial position and operations that present separately the financial results of NYPH and the Consolidated Entities. The Three Months Ended March 31, 2025 Unaudited Financial Statements include all adjustments, consisting of normal recurring accruals, which the Hospital considered necessary for a fair presentation of the financial position and the results of operations for these periods. Certain amounts included in the Three Months Ended March 31, 2025 Unaudited Financial Statements are recorded based on annual estimates, are only updated annually, or are projected for interim financial reporting purposes; however, such amounts and estimates are subject to change and are reevaluated by the Hospital monthly and on an annual basis. The results for the three months ended March 31, 2025 are not necessarily indicative of the results that may be expected for the entire year ending December 31, 2025.

The financial results of NYPH and the Consolidated Entities, including, without limitation, NYP Community Programs, NYP Hudson Valley and NYP Queens, are reflected in the consolidated financial statements of the Hospital. **However, the financial results and utilization data of the Consolidated Entities are not reflected in the information presented in this Disclosure Report. None of the Consolidated Entities is a Member of the Obligated Group. None of the Consolidated Entities has any financial or other obligation under the Master Trust Indenture or any Bond Indenture to which NYPH is a party. None of the assets or revenue of the Consolidated Entities, NYP Community Programs, NYP Hudson Valley or NYP Queens is legally committed to the repayment of any of the debt of NYPH.**

¹ As discussed on page 1, the term "the Hospital" is used in this Disclosure Report to refer to NYPH and its Consolidated Entities collectively. The term "NYPH" is used to refer to The New York and Presbyterian Hospital as a legal entity separate from the Consolidated Entities.

SUMMARY STATEMENTS OF OPERATIONS OF NYPH

(\$ in thousands)

	Unaudited	
	Quarters Ended	
	03/31/25	03/31/24
Operating Revenues		
Net patient service revenue	\$ 2,514,806	\$ 2,477,575
Other revenue ¹	197,898	142,961
Total operating revenues	<u>2,712,704</u>	<u>2,620,536</u>
Operating Expenses		
Salaries and wages	1,222,901	1,166,729
Employee benefits	324,462	300,522
Supplies and other expenses	900,359	877,962
Interest expense, net of amortization	34,475	34,727
Depreciation and amortization	133,626	128,114
Total operating expenses	<u>2,615,823</u>	<u>2,508,054</u>
Operating income	96,881	112,482
Investment return – net	14,094	274,025
Expected return on plan assets and other components of net periodic pension and postretirement cost	15,450	13,629
Excess of revenues over expenses	<u>126,425</u>	<u>400,136</u>
Other changes in net assets without donor restrictions:		
Net asset transfers to related parties	(3,712)	–
Other changes in net assets	(3)	(1)
Distributions from New York-Presbyterian Fund, Inc. for the purchase of fixed assets	25,146	4,222
Change in pension and postretirement benefit liabilities to be recognized in future periods	(27,531)	15,563
Change in net assets without donor restrictions	<u>\$ 120,325</u>	<u>\$ 419,920</u>

Source: Three Months Ended March 31, 2025 Unaudited Financial Statements. Amounts in the table are for NYPH only and exclude amounts for the Consolidated Entities.

- For the three months ended March 31, 2025, other revenue included \$30.0 million recognized related to a grant from the Federal Emergency Management Agency ("FEMA") that was approved for reimbursement in January 2025 in a total amount of \$510.3 million and pending final review procedures by the State of New York Division of Homeland Security and Emergency Services; the payment to the Hospital was not been received through March 31, 2025; approximately \$295.0 million in relation to this grant was received in May 2025. There were no FEMA receipts included in other revenue for the three months ended March 31, 2024.

SUMMARY STATEMENTS OF FINANCIAL POSITION OF NYPH
(\$ in thousands)

	<u>Unaudited</u> <u>March 31,</u> <u>2025</u>	<u>Audited</u> <u>December 31,</u> <u>2024</u>
Assets		
Current assets:		
Cash, cash equivalents and short-term investments:		
Cash and cash equivalents	\$ 502,916	\$ 597,328
Short-term investments	2,645,392	2,624,175
Total cash, cash equivalents and short-term investments	<u>3,148,308</u>	<u>3,221,503</u>
 Patient accounts receivable – net	 1,481,430	 1,531,274
Other current assets	569,419	502,557
Assets limited as to use – current portion	54,831	53,548
Professional liabilities insurance recoveries receivable and related deposits – current portion	 86,442	 86,442
Beneficial interest in net assets held by related organizations – current portion	 73,141	 79,023
Due from related organizations – net	507,472	498,938
Loans receivable from Regional Hospitals – current portion	9,327	9,230
Total current assets	<u>5,930,370</u>	<u>5,982,515</u>
 Assets limited as to use – noncurrent	 6,993,846	 6,966,547
Property, buildings and equipment – net	4,747,393	4,656,239
Operating lease assets	389,966	400,399
Other noncurrent assets – net	178,491	220,425
Loans receivable from Regional Hospitals – net of current portion	180,498	182,866
Professional liabilities insurance recoveries receivable and related deposits – noncurrent	 347,470	 347,470
Beneficial interest in net assets held by related organizations – noncurrent	 3,051,437	 3,062,838
Total assets	<u><u>\$ 21,819,471</u></u>	<u><u>\$ 21,819,299</u></u>

Source: Three Months Ended March 31, 2025 Unaudited Financial Statements and 2024 Audited Financial Statements. Amounts in the table are for NYPH only and exclude amounts for the Consolidated Entities.

SUMMARY STATEMENTS OF FINANCIAL POSITION OF NYPH (continued)
(\$ in thousands)

	<u>Unaudited</u> <u>March 31,</u> <u>2025</u>	<u>Audited</u> <u>December 31,</u> <u>2024</u>
Liabilities and net assets		
Current liabilities:		
Long-term debt – current portion	\$ 74,959	\$ 75,608
Operating lease liability – current portion	42,596	44,490
Accounts payable and accrued expenses	823,379	809,182
Accrued salaries and related liabilities	678,545	679,736
Postretirement benefit liability – current portion	3,623	3,623
Professional and other insurance liabilities – current portion	104,705	103,651
Other current liabilities	529,645	435,787
Total current liabilities	<u>2,257,452</u>	<u>2,152,077</u>
Long-term debt	3,749,633	3,756,691
Operating lease liability	370,682	379,473
Professional and other insurance liabilities	851,729	854,094
Postretirement benefit liability	26,630	26,148
Other noncurrent liabilities	462,585	653,098
Total liabilities	<u>7,718,711</u>	<u>7,821,581</u>
Net assets:		
Net assets without donor restrictions	10,962,506	10,842,181
Net assets with donor restrictions	3,138,254	3,155,537
Total net assets	<u>14,100,760</u>	<u>13,997,718</u>
Total liabilities and net assets	<u><u>\$ 21,819,471</u></u>	<u><u>\$ 21,819,299</u></u>

Source: Three Months Ended March 31, 2025 Unaudited Financial Statements and 2024 Audited Financial Statements. Amounts in the table are for NYPH only and exclude amounts for the Consolidated Entities.

Operating Margin

The following table sets forth the total operating margin of NYPH based on total operating revenue and operating income for the three months ended March 31, 2025 and the year ended December 31, 2024. Amounts in the table are for NYPH only and exclude amounts for the Consolidated Entities.

OPERATING MARGIN OF NYPH

(\$ in thousands)

	As of March 31, 2025	As of December 31, 2024
Total operating revenue	\$2,712,704	\$10,640,842
Operating income	\$96,881	\$445,282
Total operating margin	3.6%	4.2%

Source: Three Months Ended March 31, 2025 Unaudited Financial Statements and 2024 Audited Financial Statements

Liquidity

The following table sets forth NYPH's days cash on hand based on unrestricted cash and investments and average daily operating expenses as of March 31, 2025 and December 31, 2024. Amounts in the table are for NYPH only and exclude amounts for the Consolidated Entities.

DAYS CASH ON HAND OF NYPH

(\$ in thousands)

	As of March 31, 2025	As of December 31, 2024
Unrestricted cash and investments ¹	\$9,725,157	\$9,808,013
Average daily operating expenses ²	\$27,580	\$26,463
Days cash on hand³	352.6	370.6

Source: NYPH records

1. Amounts include all cash and cash equivalents, short-term investments, funded depreciation investments, and investments made with proceeds of the Series 2015 Bonds, Series 2016 Bonds, Series 2019 Bonds and Series 2020 Bonds (each, as defined below under "Outstanding Long-Term Indebtedness – The MTI Indebtedness"), but exclude any donor restricted funds and other third-party restricted funds.
2. Total operating expenses for the period less depreciation and amortization divided by 90 and 366 for the three months ended March 31, 2025 and the year ended December 31, 2024, respectively.
3. Unrestricted cash and investments divided by average daily operating expenses.

Long-Term Debt Service Coverage

The following table sets forth the debt service coverage for NYPH for the three months ended March 31, 2025 and the year ended December 31, 2024 based on (i) historical maximum annual debt service, and (ii) actual annual debt service. The amounts in the table are for NYPH only and exclude amounts for the Consolidated Entities.

LONG-TERM DEBT SERVICE COVERAGE OF NYPH

(\$ in thousands)

	Three Months Ended March 31, 2025	Year Ended December 31, 2024
Income available for debt service¹:		
Change in net assets without donor restrictions	\$120,325	\$1,576,548
Depreciation and amortization	133,626	510,080
Interest expense, net of amortization	34,475	139,338
Distribution from New York-Presbyterian Fund, Inc. for the purchase of fixed assets	(25,146)	(60,378)
Actuarial loss from pension termination settlements or curtailments	—	454
Change in unrealized gains and losses on investment and change in interest in HERS	(10,331)	(896,908)
Equity in earnings on alternative investments and investment return of captive insurance companies	514	(23,310)
Change in pension and post-retirement benefit liabilities to be recognized in future periods	27,531	(72,157)
Total	\$280,994	\$1,173,667
 Annualized Total ²	 \$1,123,976	
 Historical maximum annual debt service ³	 \$254,554	 \$254,554
 Long-Term Debt Service Coverage (MADS)	 4.42x	 4.61x
 Actual annual debt service ⁴	 \$219,058	 \$209,854
 Long-Term Debt Service Coverage (Actual)	 5.13x	 5.59x

Source: NYPH records and 2024 Audited Financial Statements.

- Income available for debt service is determined in accordance with the Master Trust Indenture. Line items not relevant to these periods are omitted.
- Results for the three months ended March 31, 2025 are annualized.
- Maximum annual debt service assumes level debt service on the Series 2015, Series 2016, Series 2019 and Series 2020 Bonds to their respective maturity dates. In fact, the Series 2015 Bonds require no payment of principal until their maturity in 2045 and the Series 2016 Bonds require no payment of principal until the maturity of their first tranche in 2036. The Series 2019 Bonds require no payment of principal until their maturity in 2119. The Series 2020 Bonds require no payment of principal until the maturity of their first tranche in 2030. All other long-term indebtedness has a maturity of less than 30 years and the scheduled amortization on such indebtedness is used for determining maximum annual debt service.
- Actual annual debt service for the calculation of Long-Term Debt Service Coverage for the three months ended March 31, 2025 takes into account all regularly scheduled payments on long-term indebtedness for the year ending December 31, 2025. For the year ended December 31, 2024, actual annual debt service takes into account all regularly scheduled payments on long-term indebtedness for the period.

Capitalization

The following table sets forth the historical capitalization of NYPH as of March 31, 2025 and as of December 31, 2024. The table is for NYPH only and excludes debt and assets of the Consolidated Entities.

CAPITALIZATION OF NYPH

(\$ in thousands)

	Actual March 31, 2025	Actual December 31, 2024
Long-term debt:		
Bonds	\$3,142,900	\$3,142,900
Other long-term debt ¹	<u>685,023</u>	<u>692,259</u>
Total long-term debt ²	3,827,923	3,835,159
Less: Current portion of long-term debt ^{1,2}	<u>(73,074)</u>	<u>(73,723)</u>
Long-term debt net of current portion ¹	3,754,849	3,761,436
Net assets without donor restrictions	<u>10,962,506</u>	<u>10,842,181</u>
Total capitalization	\$14,717,355	\$14,603,617
Long-term debt to total capitalization	25.5%	25.8%

Source: NYPH records

1. Includes the Promissory Note and Private Placement Note (each, as defined in "Outstanding Long-Term Indebtedness – Other Indebtedness"), term loan payable, and finance leases, and excludes unamortized deferred financing costs and bond premium which are reported net in current and long-term debt.
2. The total long-term debt excludes deferred financing costs of \$35.6 million and \$36.0 million as of March 31, 2025 and December 31, 2024, respectively. The total long-term debt excludes bond premium of \$32.2 million and \$33.1 million as of March 31, 2025 and December 31, 2024, respectively. The current portion of long-term debt excludes deferred financing costs of approximately \$1.6 million as of March 31, 2025 and December 31, 2024, and bond premium of approximately \$3.5 million as of March 31, 2025 and December 31, 2024.

Cash to Debt

The following table sets forth NYPH's unrestricted cash and investments to long-term debt as of March 31, 2025 and December 31, 2024. The table is for NYPH only and excludes debt and unrestricted cash and investments of the Consolidated Entities.

CASH TO DEBT OF NYPH (\$ in thousands)

	Actual March 31, 2025	Actual December 31, 2024
Unrestricted cash and investments ¹	\$9,725,157	\$9,808,013
Long-term debt:		
Bonds	\$3,142,900	\$3,142,900
Other long-term debt ²	<u>685,023</u>	<u>692,259</u>
Total long-term debt ³	3,827,923	3,835,159
Less: Current portion of long-term debt ^{2,3}	<u>(73,074)</u>	<u>(73,723)</u>
Net long-term debt	\$3,754,849	\$3,761,436
Unrestricted cash and investments to long-term debt	259.0%	260.8%

Source: NYPH records

1. Amounts as of include all cash and cash equivalents, short-term investments, funded depreciation investments, and investments made with proceeds of the Series 2015 Bonds, Series 2016 Bonds, Series 2019 Bonds and Series 2020 Bonds (each, as defined below under "Outstanding Long-Term Indebtedness – The MTI Indebtedness"), but exclude any donor restricted funds and other third-party restricted funds.
2. Includes the Promissory Note and Private Placement Note (each, as defined in "Outstanding Long-Term Indebtedness – Other Indebtedness"), term loan payable, and finance leases, and excludes unamortized deferred financing costs which are reported net in current and long-term debt.
3. The total long-term debt excludes deferred financing costs of \$35.6 million and \$36.0 million as of March 31, 2025 and December 31, 2024, respectively. The total long-term debt excludes bond premium of \$32.2 million and \$33.1 million as of March 31, 2025 and December 31, 2024, respectively. The current portion of long-term debt excludes deferred financing costs of approximately \$1.6 million as of March 31, 2025 and December 31, 2024 and bond premium of approximately \$3.5 million as of March 31, 2025 and December 31, 2024.

Management Discussion and Analysis

Utilization

Three months ended March 31, 2025 compared to three months ended March 31, 2024

For the three months ended March 31, 2025, NYPH's inpatient discharges decreased 925 cases or 2.6% compared to the same period in 2024; outpatient clinic visits increased 15,561 or 8.7% compared to the same period in 2024; emergency room visits increased 2,815 or 2.8% compared to the same period in 2024; ambulatory surgery procedures increased 1,649 or 4.7% compared to the same period in 2024; and mental health clinic visits increased 1,727 or 6.0% compared to the same period in 2024 as result of several strategic initiatives aimed at expanding access to care. The decrease in inpatient utilization during the three months ended March 31, 2025 was a result of the continuing shift in the healthcare industry from inpatient care to outpatient care, along with some physician departures. The increases in outpatient utilization during the three months ended March 31, 2025 was the result of the continuing recovery in New York City from the pandemic and continuing efforts to improve access to care. Additionally, normal planned strategic growth activities including expanding services have contributed to increasing volumes.

Recent Financial Performance

Three months ended March 31, 2025 compared to three months ended March 31, 2024

For the three months ended March 31, 2025, NYPH had an operating income of \$96.9 million, a \$15.6 million decrease from the \$112.5 million operating income for the same period in 2024. This decrease in operating results was primarily due to the decrease in inpatient volume and acuity, partially offset by increases in outpatient utilization as noted above. This decrease to operating income was partially offset by recognizing \$30.0 million of grant revenue from the FEMA Disaster Relief Fund during the three months ended March 31, 2025; there was no revenue recognized from the FEMA Disaster Relief Fund grants during the three months ended March 31, 2024.

For the three months ended March 31, 2025, NYPH had an excess of revenues over expenses of \$126.4 million, as compared to an excess of revenues over expenses of \$400.1 million for the same period in 2024. The \$273.7 million or 68.4% decrease in excess of revenues over expenses was primarily due to a decrease in investment return of \$259.9 million and the decrease in operating income of \$15.6 million for the three months ended March 31, 2025, as described above, compared to the same period in 2024. These decreases were partially offset by an increase in non-operating pension items of \$1.8 million in the three months ended March 31, 2025, compared with the same period in 2024.

For the three months ended March 31, 2025, net patient service revenue increased \$37.2 million or 1.5% compared to the same period in 2024. This increase was primarily driven by increases in outpatient volumes across all patient services, and increases in payment rates, partially offset by decreases in inpatient volume and acuity, as noted above.

For the three months ended March 31, 2025, other revenue increased \$54.9 million or 38.4% compared to the same period in 2024, as a result of recognizing \$30.0 million of grant revenue from the FEMA Disaster Relief Fund; there was no revenue recognized from FEMA grants during the comparable period in 2024. Additionally, there were increases in specialty pharmacy revenue of \$23.6 million, distributions from the radiology joint venture of \$2.5 million and vendor rebates of \$2.0 million. These increases were partially offset by decreases in distributions from real estate operations of \$2.5 million and net assets released from restrictions of \$0.7 million in the three months ended March 31, 2025, as compared to the three months ended March 31, 2024.

For the three months ended March 31, 2025, total operating expenses increased \$107.8 million or 4.3% compared with the same period in 2024. Salaries and wages, together with employee benefits, increased \$80.1 million or 5.5% over the same period in 2024. This increase, compared to the three months ended March 31, 2024, was primarily a result of having 812 more full-time equivalent employees, and general wage and benefits increases consistent with the labor market. These increases were partially offset by reduced utilization of temporary and agency help and reduced hourly rates for agency nurses, resulting in a \$2.1 million or 8.0% decrease in temporary and agency costs.

For the three months ended March 31, 2025, supplies and other expenses increased \$22.4 million or 2.6% over the same period in the previous year. This increase was primarily due to an increase in costs associated with providing services to treat patients, partially offset by recording lower costs to reflect estimates for professional liabilities and related exposures as compared to the three months ended March 31, 2024. Depreciation and amortization expense increased \$5.5 million or 4.3% as the result of projecting an increase in completing more capital projects during the three months ended March 31, 2025, compared to the same period in 2024.

For the three months ended March 31, 2025, NYPH's change in net assets without donor restrictions totaled \$120.3 million compared to \$419.9 million in the same period in 2024, a decrease of \$299.6 million. This was primarily due to a decrease in investment return of \$259.9 million, a decrease to the change in the pension and postretirement benefits liabilities of \$43.1 million, decreases in operating income of \$15.6 million and an increase of net asset transfers to related parties of \$3.7 million in 2025, compared to the same period in 2024. These decreases to net assets without donor restrictions were partially offset by higher distributions from Fund, Inc. for the purchase of fixed assets, which increased by \$20.9 million, and an increase in non-operating pension items of \$1.8 million for the three months ended March 31, 2025, compared to the same period in 2024.

Settlement of Litigation related to Robert Hadden

Supplementing Note 12 to the 2024 Audited Financial Statements and Note 6 to the Three Months Ended March 31, 2025 Unaudited Consolidated Financial Statements, in early May 2025, a final settlement was announced for litigation against Columbia University and NYPH by patients alleging sexual abuse by Robert Hadden, a physician formerly employed by Columbia University's medical school, Columbia VP&S. Payments to be made by NYPH in relation to this and earlier settlements of Hadden claims are within the range it estimated and recorded previously on this matter. While NYPH is named as a defendant in this litigation and in other suits and claims involving a former faculty member employed by one of the Medical Schools, NYPH does not necessarily have responsibility or liability for the acts of a physician employed by one of the Medical Schools.

Outstanding Long-Term Indebtedness

Debt Structure

NYPH's long-term indebtedness outstanding as of the date of this Disclosure Report can be categorized into two groups: (1) the Master Trust Indenture (MTI) Indebtedness, and (2) Other Indebtedness.

The following table sets forth the long-term indebtedness of NYPH as of March 31, 2025 and December 31, 2024. The table is for NYPH only and excludes the long-term indebtedness of the Consolidated Entities.

OUTSTANDING LONG-TERM INDEBTEDNESS OF NYPH (\$ in thousands)

Indebtedness	Maturity Date	Outstanding at	
		March 31, 2025	December 31, 2024
MTI			
Series 2015 Bonds (fixed rate, taxable)	2045	750,000	750,000
Series 2016 Bonds (fixed rate, taxable)	2036	250,000	250,000
Series 2016 Bonds (fixed rate, taxable)	2056	350,000	350,000
Series 2016 Bonds (fixed rate, taxable)	2116	250,000	250,000
Series 2019 Bonds (fixed rate, taxable)	2119	500,000	500,000
Series 2020 Bonds (fixed rate, taxable)	2030	150,000	150,000
Series 2020 Bonds (fixed rate, taxable)	2040	300,000	300,000
Series 2020 Bonds (fixed rate, taxable)	2060	300,000	300,000
Series 2023 Bonds (fixed rate, tax exempt)	2038	292,900	292,900
Other			
Promissory Note - 466 Lexington Avenue	2048	281,945	282,215
Private Placement Note	2047	184,000	184,000
Term loan payable - NYP Westchester	2025	1,503	2,153
Finance leases	Various	217,575	223,891
Total		\$ 3,827,923	\$ 3,835,159
Add: Unamortized bond premium		32,248	33,127
Less: Deferred financing costs, net of accumulated amortization		35,579	35,987
Total		\$ 3,824,592	\$ 3,832,299

Source: NYPH records

The MTI Indebtedness

Indebtedness evidenced by an Obligation issued under the Master Trust Indenture constitutes a joint and several obligation of the Members of the Obligated Group (as defined in the Master Trust Indenture), including any entity that may in the future become a Member of the Obligated Group, subject to the right of a Member to withdraw from the Obligated Group upon meeting certain conditions set forth in the Master Trust Indenture. Currently, NYPH is the only Member of the Obligated Group, and has agreed not to withdraw from the Obligated Group while the NYPH Taxable Bonds, Series 2015, Series 2016, Series 2019, Series 2020 (respectively, the “Series 2015 Bonds”, the “Series 2016 Bonds”, the “Series 2019 Bonds”, and the “Series 2020 Bonds”) or The New York and Presbyterian Hospital Obligated Group Revenue Bonds, Series 2023A (the “Series 2023 Bonds”) are outstanding. There are currently five Obligations issued under the Master Trust Indenture: (i) Obligation No. 1, which was issued to the bond trustee for the Series 2015 Bonds, (ii) Obligation No. 2, which was issued to the bond trustee for the Series 2016 Bonds, (iii) Obligation No. 3, which was issued to the bond trustee for the Series 2019 Bonds, (iv) Obligation No. 4, which was issued to the bond trustee for Series 2020 Bonds, and (v) Obligation No. 5 which was issued to the bond trustee for the Series 2023 Bonds.

The table below sets forth each of the Obligations currently outstanding, the Series of Bonds to which such Obligation relates, and its principal amount and maturity date.

MTI INDEBTEDNESS

(\$ in thousands)

<u>Obligation</u>	<u>Debt Evidenced</u>	<u>Principal Amount</u>	<u>Maturity</u>
No. 1	Series 2015 Bonds	\$ 750,000	August 1, 2045
No. 2	Series 2016 Bonds	850,000	August 1, 2116 ¹
No. 3	Series 2019 Bonds	500,000	August 1, 2119
No. 4	Series 2020 Bonds	750,000	August 1, 2060 ²
No. 5	Series 2023 Bonds	292,900	August 1, 2038 ³
Total		<u>\$ 3,142,900</u>	

Source: NYPH records

1. While Obligation No. 2 matures in 2116, principal payments are due in the following amounts on August 1 of the years indicated: \$250.0 million in 2036, \$350.0 million in 2056 and \$250.0 million in 2116.
2. While Obligation No. 4 matures in 2060, principal payments are due in the following amounts on August 1 of the years indicated: \$150.0 million in 2030, \$300.0 million in 2040, and \$300.0 million in 2060.
3. While Obligation No. 5 matures in 2038, principal payments are due annually beginning August 1, 2029 in amounts that match the principal amounts due on the Series 2023 Bonds on such dates.

Other Indebtedness

On March 10, 2022, NYPH entered into a private placement Note Purchase Agreement which authorized the issuance and sale of \$200.0 million aggregate principal amount of Senior Notes (collectively, the "Private Placement Note"). The closing of the sale and the purchase of the Private Placement Note was completed in May 2022 and the maturity date of the Private Placement Note is May 2047. The Private Placement Note is an unsecured general obligation of NYPH. The Private Placement Note is outstanding in the aggregate principal amount of \$184.0 million as of March 31, 2025. The Note Purchase Agreement grants NYPH the right to elect at any time to issue Obligations under the Master Trust Indenture to evidence its payment obligations under the Private Placement Note, subject to certain conditions.

NYPH has various finance leases totaling \$217.6 million as of March 31, 2025, which are secured by the financed equipment.

NYPH has three unsecured lines of credit: (i) a \$350.0 million line of credit agreement with a commercial bank available through November 30, 2027; (ii) a \$300.0 million line of credit with a second commercial bank available through November 30, 2027; and (iii) a \$300.0 million line of credit with a third commercial bank available through December 21, 2027. As of the date of this Disclosure Report, there is no outstanding balance on these three credit facilities, which provide a total of \$950.0 million in availability. There were no draws on the lines of credit during 2024 or 2025 to the date of this Disclosure Report.

In June 2017, NYPH entered into a transaction pursuant to which NYPH purchased a 30-year leasehold condominium interest (with an option to extend) in approximately 480,000 square feet of space located at 466 Lexington Avenue, New York (the "Leasehold Condominium") to consolidate corporate services of the Hospital. To finance the acquisition of the Leasehold Condominium, NYPH issued to the seller a promissory note in the principal amount of \$249.9 million (the "Promissory Note") which bears interest at a rate of 7% per annum. Interest payments for the period from July 2017 through December 2018 were deferred. The debt service terms require principal and interest payments in escalating amounts over the life of the note, ranging from \$1.5 million to \$2.5 million commencing January 2019 through December 2048. Unpaid interest incurred through December 31, 2023 accrued to the outstanding principal balance on the Promissory Note. As of March 31, 2025, the outstanding balance due includes the principal amount of the Promissory Note and accrued interest, totaling \$281.9 million. The Promissory Note is secured by a mortgage granted by NYPH in its interest in the Leasehold Condominium. In connection with this transaction, the seller/landlord provided NYPH with a tenant allowance for leasehold improvements of approximately \$75.6 million. As of March 31, 2025, the unamortized balance remaining is \$57.0 million, which was recorded in the Hospital's consolidated statement of financial position.

On April 1, 2018, in connection with the merger of Lawrence Hospital into NYPH, NYPH assumed a term loan made by a commercial bank. This loan is an unsecured general obligation of NYPH which matures on October 1, 2025. The outstanding principal amount at March 31, 2025 was \$1.5 million.

NYPH has a reimbursement agreement in place to support three letters of credit in an aggregate amount of \$56.9 million. The letters of credit were issued on behalf of NYPH in connection with its workers' compensation program. As of the date of this Disclosure Report, no draws have been made under the letters of credit.

Investments

The following is a summary table of NYPH's cash and investment assets as of March 31, 2025, which includes NYPH's beneficial interest in net assets held by related organizations (NYPH's interest in assets held by Fund, Inc.) and assets attributable to NYPH's member capital account held in Hudson East River Systems, LLC ("HERS"), a pooled investment program through a limited liability company of which Fund, Inc. is the managing member. The table excludes assets invested in NYPH's pension plans, as well as assets of NYPH's Consolidated Entities.

CASH AND INVESTMENT ASSETS OF NYPH

	<u>Total Investment Assets</u>	<u>Beneficial Interest¹</u>	<u>NYPH Assets²</u>
Total (in billions)	\$12.8	\$2.6	\$10.2
Cash	7%	4%	9%
Treasuries	9	3	11
Public Equity	34	45	31
Private Equity	15	20	14
Real Estate	5	6	4
Natural Resources	3	4	2
Hedge Funds	14	18	13
Credit	<u>13</u>	<u>N/A</u>	<u>16</u>
Total ³	100%	100%	100%

Source: NYPH records

1. The beneficial interest component is almost entirely comprised of NYPH's beneficial interest in Fund, Inc.

2. Includes NYPH's interest in HERS.

3. Totals may not foot due to rounding.

The above table details NYPH's investment assets, including NYPH's beneficial interest in net assets held by related organizations, which comprise the majority of the investment assets under the purview of the Investment Committee and Investment Staff. The NYPH component includes investments limited as to use, including funded depreciation, and funded self-insurance. The NYPH component also includes short-term and cash equivalent investments maintained for purposes of operations and other needs of NYPH, escrow funds, and other assets. The beneficial interest component is almost entirely comprised of NYPH's beneficial interest in Fund, Inc. The table does not include investments held by NYPH's pension plans, which are invested similarly to other long-term assets and assets of NYPH's Consolidated Entities.

For long-term funds, NYPH has adopted a return-oriented, well-diversified asset allocation strategy.

The following table shows, as of March 31, 2025, the liquidity of NYPH's investment portfolio, with 53% or \$6.8 billion of assets accessible within one month. The table excludes assets of NYPH's pension plans and assets of NYPH's Consolidated Entities.

LIQUIDITY OF NYPH'S INVESTMENT PORTFOLIO

	<u>NYPH Long-Term Fund</u>	<u>NYPH Current Investments</u>	<u>Total</u>
1-5 Days	30%	100%	50%
6-31 Days	3	N/A	3
1-6 Months	12	N/A	8
7-12 Months	8	N/A	6
1-3 Years	10	N/A	7
3+ Years	<u>37</u>	<u>N/A</u>	<u>26</u>
Total ¹	100%	100%	100%

Source: NYPH records

1. Totals may not foot due to rounding.

The following tables show (i) the amount of investment assets of NYPH under management as of March 31, 2025, and as of December 31, 2024, and (ii) the investment performance of the long-term portfolio through March 31, 2025. The table excludes assets invested in NYPH's pension plans and assets of NYPH's Consolidated Entities.

INVESTMENT ASSETS UNDER MANAGEMENT

(\$ in billions)

	<u>As of</u> <u>March 31, 2025</u>	<u>As of</u> <u>December 31, 2024</u>
Investment Assets Under Management	\$12.8	\$12.9

Source: NYPH records

LONG-TERM PORTFOLIO PERFORMANCE

	<u>5 Year</u>	<u>3 Year</u>	<u>1 Year</u>
Long -Term Portfolio Performance through March 31, 2025	9.7%	4.2%	8.0%

Source: NYPH records

Philanthropy

A campaign was launched in 2015 to raise \$300.0 million for the Alexandra Cohen Hospital for Women and Newborns. As of March 31, 2025, NYPH has received pledges and gifts of \$197.1 million related to this campaign. After a Quiet Phase, a \$2.0 billion fundraising campaign was publicly launched in October 2024. As of March 31, 2025, approximately \$1.4 billion in pledges and gifts have been made to this campaign. The collection rate for pledges, based on recent history, exceeds 95%.

Fund, Inc.

During the three months ended March 31, 2025 and the year ended December 31, 2024, Fund, Inc. distributed to NYPH approximately \$25.2 million and \$60.4 million, respectively, for the purchase of fixed assets. Fund, Inc. holds certain assets on behalf of NYPH that are restricted and may only be used for the benefit of NYPH. Refer to Note 7 of the 2024 Audited Financial Statements for an annual summary of the activity within the Fund, Inc. restricted net assets. There is no legal or contractual requirement that Fund, Inc. continue to make distributions of its unrestricted assets to NYPH.

Capital Expenditures

For the three months ended March 31, 2025 and the year ended December 31, 2024, NYPH incurred capital expenditures of \$224.8 million and \$727.6 million, respectively, for the acquisition of property, buildings and equipment (net of disposals). These expenditures were funded from internally generated cash flow, equipment finance leases and donations. For the three months ended March 31, 2025 and the year ended December 31, 2024, Fund, Inc. received approximately \$44.7 million and \$188.2 million, respectively, (including \$43.0 million and \$177.2 million, respectively, specifically restricted for NYPH) in new gifts and pledges from individuals, foundations and corporate donors for capital acquisitions and other purposes.

RCP Corporations

For the three months ended March 31, 2025 and the year ended December 31, 2024, the RCP Corporations had an aggregate net operating income of approximately \$7.0 million and \$33.2 million, respectively. Historically, each of the RCP Corporations has distributed its excess of revenue over expenses, less reasonable reserves ("Excess Revenue") to NYPH. For the three months ended March 31, 2025 and the year ended December 31, 2024, these amounts totaled \$6.8 million and \$32.1 million, respectively, and are included in other operating revenue in the Statements of Operations of NYPH. However, the board of directors of each of the RCP Corporations has complete discretion within its stated corporate purposes with respect to the distribution of its Excess Revenue; there is no legal or contractual requirement that these amounts be distributed to NYPH.



UNAUDITED CONSOLIDATED FINANCIAL STATEMENTS
AND SUPPLEMENTARY INFORMATION

The New York and Presbyterian Hospital

As of and For the Three Months Ended March 31, 2025

The New York and Presbyterian Hospital

Unaudited Consolidated Financial Statements and Supplementary Information

As of and For the Three Months Ended March 31, 2025

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The New York and Presbyterian Hospital

Unaudited Consolidated Statements of Financial Position (In Thousands)

	(Unaudited) March 31 2025	(Audited) December 31 2024
Assets		
Current assets:		
Cash, cash equivalents and short-term investments:		
Cash and cash equivalents	\$ 625,905	\$ 730,330
Short-term investments	2,752,131	2,730,487
Total cash, cash equivalents and short-term investments	3,378,036	3,460,817
Patient accounts receivable – net	1,705,550	1,756,665
Other current assets	728,427	638,614
Assets limited as to use – current portion	56,609	55,342
Professional liabilities insurance recoveries receivable and related deposits – current portion	101,363	101,363
Beneficial interest in net assets held by related organizations – current portion	73,141	79,023
Due from related organizations – net	48,101	70,484
Total current assets	6,091,227	6,162,308
Assets limited as to use – noncurrent	7,076,998	7,049,773
Property, buildings and equipment – net	5,320,780	5,224,673
Operating lease assets	468,876	480,641
Other noncurrent assets – net	207,403	247,980
Professional liabilities insurance recoveries receivable and related deposits – noncurrent	415,637	415,637
Beneficial interest in net assets held by related organizations – noncurrent	3,051,437	3,062,838
Total assets	\$ 22,632,358	\$ 22,643,850

Continued on following page.

The New York and Presbyterian Hospital

Unaudited Consolidated Statements of Financial Position (continued) (In Thousands)

	(Unaudited) March 31 2025	(Audited) December 31 2024
Liabilities and net assets		
Current liabilities:		
Long-term debt – current portion	\$ 77,297	\$ 77,893
Operating lease liability – current portion	57,936	60,266
Accounts payable and accrued expenses	1,077,378	1,072,792
Accrued salaries and related liabilities	783,931	774,801
Postretirement benefit liability – current portion	4,694	4,694
Professional and other insurance liabilities – current portion	120,478	119,424
Other current liabilities	539,567	445,223
Total current liabilities	2,661,281	2,555,093
Long-term debt	3,754,974	3,759,592
Operating lease liability	437,242	446,983
Professional and other insurance liabilities	977,437	977,282
Postretirement benefit liability	42,589	42,409
Other noncurrent liabilities	485,401	684,404
Total liabilities	8,358,924	8,465,763
Commitments and contingencies		
Net assets:		
Net assets without donor restrictions	11,109,353	10,997,313
Net assets with donor restrictions	3,164,081	3,180,774
Total net assets	14,273,434	14,178,087
Total liabilities and net assets	\$ 22,632,358	\$ 22,643,850

See accompanying notes.

The New York and Presbyterian Hospital

Unaudited Consolidated Statements of Operations
(In Thousands)

(Unaudited)

Three Months Ended March 31

2025 2024

Operating revenues

Net patient service revenue	\$ 2,944,983	\$ 2,884,705
Other revenue	458,384	339,562
Total operating revenues	<u>3,403,367</u>	<u>3,224,267</u>

Operating expenses

Salaries and wages	1,508,207	1,430,430
Employee benefits	404,164	374,624
Supplies and other expenses	1,219,995	1,138,983
Interest expense, net of amortization	36,007	36,348
Depreciation and amortization	148,831	142,904
Total operating expenses	<u>3,317,204</u>	<u>3,123,289</u>

Operating income	86,163	100,978
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Investment return, net	15,331	280,375
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Expected return on plan assets and other components of net periodic pension and postretirement cost	16,783	14,870
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Excess of revenues over expenses	<u>118,277</u>	<u>396,223</u>
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Other changes in net assets without donor restrictions:

Net asset transfers to related parties	(3,712)	—
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Net asset transfers from New York-Presbyterian Healthcare System, Inc.	—	4,607
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Other changes in net assets	(140)	(74)
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Distributions from New York-Presbyterian Fund, Inc. for the purchase of fixed assets	25,146	4,222
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Change in pension and postretirement benefit liabilities to be recognized in future periods	(27,531)	15,563
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Change in net assets without donor restrictions	<u>\$ 112,040</u>	<u>\$ 420,541</u>
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See accompanying notes.

The New York and Presbyterian Hospital

Unaudited Consolidated Statements of Changes in Net Assets (In Thousands)

	Net Assets		Beneficial Interest in Net Assets with Donor					Total Net Assets with Donor Restrictions	Total Net Assets
	without Donor Restrictions	Net Assets with Donor Restrictions	Restrictions Held by Related Organizations				Total Beneficial Interest		
			Plant Replacement	Specific Purposes	Endowment Earnings	Permanent Endowment			
(Unaudited)									
Net assets at January 1, 2024	\$ 9,434,393	\$ 36,267	\$ 1,049,434	\$ 1,243,399	\$ 239,320	\$ 305,400	\$ 2,837,553	\$ 2,873,820	\$ 12,308,213
Change in net assets without donor restrictions	420,541	—	—	—	—	—	—	—	420,541
Net investment return - donor restricted	—	1,408	—	—	—	—	—	1,408	1,408
Changes in beneficial interest in net assets held by related organizations	—	—	28,112	34,185	11,065	4,219	77,581	77,581	77,581
Change in net assets	420,541	1,408	28,112	34,185	11,065	4,219	77,581	78,989	499,530
Net assets at March 31, 2024	<u>\$ 9,854,934</u>	<u>\$ 37,675</u>	<u>\$ 1,077,546</u>	<u>\$ 1,277,584</u>	<u>\$ 250,385</u>	<u>\$ 309,619</u>	<u>\$ 2,915,134</u>	<u>\$ 2,952,809</u>	<u>\$ 12,807,743</u>
Net assets at January 1, 2025	\$ 10,997,313	\$ 38,913	\$ 1,202,837	\$ 1,352,604	\$ 268,059	\$ 318,361	\$ 3,141,861	\$ 3,180,774	\$ 14,178,087
Change in net assets without donor restrictions	112,040	—	—	—	—	—	—	—	112,040
Net investment return - donor restricted	—	590	—	—	—	—	—	590	590
Changes in beneficial interest in net assets held by related organizations	—	—	(22,110)	7,605	(3,959)	1,181	(17,283)	(17,283)	(17,283)
Change in net assets	112,040	590	(22,110)	7,605	(3,959)	1,181	(17,283)	(16,693)	95,347
Net assets at March 31, 2025	<u>\$ 11,109,353</u>	<u>\$ 39,503</u>	<u>\$ 1,180,727</u>	<u>\$ 1,360,209</u>	<u>\$ 264,100</u>	<u>\$ 319,542</u>	<u>\$ 3,124,578</u>	<u>\$ 3,164,081</u>	<u>\$ 14,273,434</u>

See accompanying notes.

The New York and Presbyterian Hospital

Unaudited Consolidated Statements of Cash Flows

(In Thousands)

	(Unaudited)	
	Three Months Ended March 31	
	2025	2024
Operating activities		
Change in net assets	\$ 95,347	\$ 499,530
Adjustments to reconcile change in net assets to net cash provided by operating activities:		
Depreciation and amortization	148,831	142,904
Amortization of bond premium, deferred financing costs and other	(448)	(448)
Net asset transfer from New York-Presbyterian Healthcare System, Inc.	—	(4,607)
Distributions from New York-Presbyterian Fund, Inc. for the purchase of fixed assets	(25,146)	(4,222)
Net investment return-without donor restrictions, excluding interest and dividends	(10,215)	(274,143)
Equity in income from investee	(195)	(744)
Net asset transfers to related parties	3,712	—
Changes in operating assets and liabilities:		
Patient accounts receivable – net	51,115	11,892
Other assets and operating lease assets	(35,727)	(34,744)
Beneficial interest in net assets held by related organizations	17,283	(77,581)
Accounts payable and accrued expenses	4,586	(27,249)
Accrued salaries and related liabilities	9,130	(90,388)
Due from related organizations – net	22,383	2,356
Other liabilities	(116,730)	95,678
Professional and other insurance liabilities and related insurance recoveries receivable and deposits	1,209	11,941
Pension and postretirement benefit liabilities	180	270
Net cash provided by operating activities	165,315	250,445
Investing activities		
Net purchases of short-term investments and assets limited as to use	(40,128)	(1,650)
Acquisitions of property, buildings and equipment – net	(236,307)	(151,640)
Distribution from investee	—	500
Loan issued to related organization	(1,549)	—
Net cash used in investing activities	(277,984)	(152,790)
Financing activities		
Repayments of long-term debt	(13,397)	(13,987)
Net asset transfer from New York-Presbyterian Healthcare System, Inc.	—	4,607
Net asset transfers to related parties	(3,712)	—
Distributions from New York-Presbyterian Fund, Inc. for the purchase of fixed assets	25,146	4,222
Net cash provided by (used in) financing activities	8,037	(5,158)
Net (decrease) increase in cash and cash equivalents and restricted cash and restricted cash equivalents	(104,632)	92,497
Cash and cash equivalents and restricted cash and restricted cash equivalents at beginning of period	806,003	692,319
Cash and cash equivalents and restricted cash and restricted cash equivalents at end of period	\$ 701,371	\$ 784,816
Supplemental disclosure of cash flow information		
Assets acquired under finance lease obligations	\$ 8,631	\$ 2,453

See accompanying notes.

The New York and Presbyterian Hospital
Notes to Unaudited Consolidated Financial Statements
March 31, 2025

1. Organization and Summary of Significant Accounting Policies

Financial Statements

The New York and Presbyterian Hospital (the Hospital) presumes that users of this unaudited consolidated financial information intended for quarterly reporting purposes have read or have access to the Hospital's audited consolidated financial statements which include certain disclosures required by accounting principles generally accepted in the United States of America (GAAP). The audited financial statements of the Hospital as of and for the years ended December 31, 2024 and 2023 are on file with the Municipal Securities Rulemaking Board and are accessible through its Electronic Municipal Market Access Database (EMMA). Accordingly, footnotes and other disclosures that would substantially duplicate the disclosures contained in the Hospital's most recent audited financial statements have been omitted from the unaudited consolidated financial information.

In the opinion of management, all adjustments considered necessary for a fair presentation of the results of the periods have been included in the accompanying unaudited consolidated financial statements. All such adjustments are considered by management to be of a normal, recurring nature, except as noted otherwise. Certain amounts pertaining to the Hospital and its subsidiaries have been reflected in the accompanying unaudited consolidated and consolidating financial information based on amounts available as of the filing date of this unaudited consolidated information and are recorded based on annual estimates, are only updated annually, or are projected for interim financial reporting purposes; however, such amounts and estimates are subject to change and are reevaluated by the Hospital quarterly and on an annual basis.

Organization and Basis of Presentation: The accompanying consolidated financial statements include the accounts of The New York and Presbyterian Hospital (referred to herein as "NYPH" when referencing the legal entity separate from its subsidiaries), NYP Community Programs, Inc. (Community Programs) and other consolidated entities as described below. NYPH is the sole member of Community Programs. The reporting entity resulting from the consolidation of these entities is referred to herein as the "Hospital." All significant intercompany balances and transactions have been eliminated in consolidation.

NYPH is a tax-exempt organization that was incorporated under New York State not-for-profit corporation law. NYPH is a major academic medical center operating at eight campuses comprised of 3,402 beds, providing a full range of inpatient and outpatient services, mainly to residents of the New York metropolitan area. The Board of Trustees of NYPH consists of persons who have first been elected as members of New York-Presbyterian Foundation, Inc. (Foundation, Inc.), a New York State not-for-profit corporation. Foundation, Inc. is related to a number of organizations.

Community Programs is the sole member of the following entities: Hudson Valley Hospital Center and Westchester Putnam Health Management System, Inc. (collectively with their controlled affiliates referred to herein as NYP/Hudson Valley or Hudson Valley); and NewYork-Presbyterian/Queens (collectively with its controlled affiliates referred to herein as NYP/Queens or Queens). NYP/Hudson Valley consists of a 128-bed acute care hospital located in Cortlandt Manor, Westchester County, New York, a fundraising foundation, a multi-specialty physician practice and certain other entities primarily formed to purchase and lease space. NYP/Queens consists of a 535-bed acute care hospital located

The New York and Presbyterian Hospital
Notes to Unaudited Consolidated Financial Statements (continued)
March 31, 2025

1. Organization and Summary of Significant Accounting Policies (continued)

in Queens County, New York, a multi-specialty physician practice and certain other entities. Hudson Valley and Queens are collectively referred to herein as the “Regional Hospitals.”

Other entities included in the accompanying consolidated financial statements include Kings Physician Services, P.C. and other physician practice and affiliated entities that comprise NYP Medical Group/Brooklyn, Lawrence Care, Inc., Lawrence Medical Associates, P.C. (d/b/a NewYork-Presbyterian Medical Group/Westchester), and NYP Sports Performance, LLC. Effective January 1, 2024, the ownership of Network Insurance Company Ltd. (NICL) was transferred from New York-Presbyterian Healthcare System, Inc., a subsidiary of Foundation, Inc., to the Hospital. NICL is a Bermuda-based captive insurance company which is being used by the Hospital for certain professional liability claims commencing January 1, 2024.

Although NYPH, Community Programs and the other entities described above have been consolidated for financial statement reporting purposes under applicable accounting requirements, there may be limitations on the use of one entity’s funds by another member of the consolidated group resulting from the charitable nature of some of the entities or other factors.

The Hospital maintains academic affiliations with two medical colleges: Columbia University Vagelos College of Physicians & Surgeons (Columbia VP&S) and The Joan and Sanford I. Weill Medical College of Cornell University (Weill Cornell Medical College) (collectively, the Schools). Transactions occur on a routine basis between the Hospital and the Schools, based upon arrangements between the parties.

Coronavirus Disease 2019 Pandemic and Related Funding

Commencing in 2020, the Hospital applied for reimbursement for qualifying expenses under the Federal Emergency Management Agency (FEMA) Disaster Relief Fund and received reimbursement payment advances in prior periods. The FEMA advances received to date relate to project worksheets totaling over \$1.0 billion submitted by the Hospital to FEMA since 2020 under its expedited claim submission process and streamlined submission process.

In January 2025, FEMA approved an obligation for reimbursement of \$510.3 million of previously submitted costs from the Hospital; the related documentation is pending final review procedures by the State of New York Division of Homeland Security and Emergency Services and the payment to the Hospital has not been received. The Hospital recognized \$30.1 million of FEMA revenue for the three months ended March 31, 2025 in relation to this approved obligation. The payment to the Hospital has not been received through March 31, 2025; approximately \$295.0 million in relation to this grant was received in May 2025.

Additionally, the Hospital is finalizing other project worksheets previously submitted to FEMA and will continue to pursue funding of eligible costs incurred through the end of the defined period. The ultimate amount of incurred costs that the Hospital may be reimbursed is still uncertain.

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

1. Organization and Summary of Significant Accounting Policies (continued)

Under the CARES Act, the Hospital is eligible to receive an employee retention credit (ERC) against the employer portion of Social Security taxes for certain wages. The ERC was designed to encourage employers to retain employees during the COVID-19 pandemic. The Hospital is eligible for ERC for the period of March 13, 2020 to September 30, 2021, as extended through further legislation. The Hospital submitted amended Forms 941-X for calendar year 2020 and the first and second quarters of calendar year 2021 to claim the ERC; the second quarter 2021 filing was submitted in February 2024 but no credit against the Hospital's taxes has been received from the Internal Revenue Service. There were no ERC received or recognized for the three months ended March 31, 2025 and 2024.

A summary of certain accounting policies follows (refer to Note 1 to the audited financial statements for further description of significant accounting policies):

Use of Estimates

The preparation of financial statements in accordance with GAAP requires management to make estimates and assumptions that affect the reported amounts of assets and liabilities, such as collections on accounts receivable for services to patients, valuation of alternative investments, estimated settlements with third-party payors, professional liabilities, and pension and postretirement benefit obligations, and disclosures of contingent assets and liabilities at the date of the consolidated financial statements. Estimates also affect the amounts of revenue and expenses reported during the period. There is at least a reasonable possibility that certain estimates will change by material amounts in the near term.

Cash and Cash Equivalents and Short-term Investments

The Hospital classifies as cash equivalents all highly liquid financial instruments with a maturity of three months or less when purchased, excluding those held in short-term investments and assets limited as to use for funded depreciation and board designated purposes as such holdings are within investment portfolios.

A reconciliation of amounts reported on the Hospital's statements of financial position to the statements of cash flows as of and for the three months ended March 31, 2025 and 2024 follows (in thousands):

	March 31,	
	2025	2024
	<i>(Unaudited)</i>	
Cash and cash equivalents	\$ 625,905	\$ 723,623
Assets limited as to use – funded self-insurance (professional liabilities): cash and cash equivalents	75,394	61,154
Assets limited as to use – donor restricted: cash and cash equivalents	72	39
Total cash and cash equivalents and restricted cash and restricted cash equivalents	\$ 701,371	\$ 784,816

The New York and Presbyterian Hospital
Notes to Unaudited Consolidated Financial Statements (continued)
March 31, 2025

1. Organization and Summary of Significant Accounting Policies (continued)

Net Patient Service Revenue

Net patient service revenue is reported at the amount that reflects the consideration to which the Hospital expects to be entitled in exchange for providing patient care. These amounts are due from patients, third-party payors (including health insurers and government programs), and others and include provisions for variable consideration (reductions to revenue) for retroactive revenue adjustments, including adjustments due to the settlement of ongoing and future audits, reviews, and investigations.

The Hospital uses a portfolio approach to account for categories of patient contracts as a collective group rather than recognizing revenue on an individual contract basis. The portfolios primarily consist of major payor classes for inpatient revenue and major types of services provided for outpatient revenue. Based on historical collection trends and other analyses, the Hospital believes that revenue recognized by utilizing the portfolio approach approximates the revenue that would have been recognized if an individual contract approach were used.

The Hospital's initial estimate of the transaction price for services provided to patients is determined by reducing the total standard charges related to the patient services provided by various elements of variable consideration, including contractual adjustments, discounts, implicit price concessions, and other reductions to the Hospital's standard charges. The Hospital determines the transaction price associated with services provided to patients who have third-party payor coverage on the basis of contractual or formula-driven rates for the services rendered. The estimates for contractual allowances and discounts are based on contractual agreements, the Hospital's discount policies and historical experience. For uninsured and under-insured patients who do not qualify for charity care, the Hospital determines the transaction price associated with services on the basis of charges reduced by implicit price concessions. Implicit price concessions included in the estimate of the transaction price are based on the Hospital's historical collection experience for applicable patient portfolios. Under the Hospital's charity care policy, a patient who has no insurance or is under-insured and is ineligible for any government assistance program has his or her bill reduced to (1) the lesser of charges or the Medicaid diagnostic-related group for inpatient and (2) a discount from Medicaid fee-for-service rates for outpatient. Patients who meet the Hospital's criteria for free care are provided care without charge; such amounts are not reported as revenue.

Generally, the Hospital bills patients and third-party payors several days after the services are performed and/or when the patient is discharged. Net patient service revenue is recognized as performance obligations are satisfied. Performance obligations are determined based on the nature of the services provided by the Hospital. Net patient service revenue for performance obligations satisfied over time is recognized based on the estimated expected payment at that point in time. The Hospital believes that this method provides a reasonable depiction of the transfer of services over the term of the performance obligation based on the services needed to satisfy the obligation. Generally, performance obligations satisfied over time relate to patients receiving inpatient acute care services or patients receiving services in the Hospital's outpatient and ambulatory care centers. The Hospital measures the performance obligation from admission into the Hospital or the commencement of an outpatient service to the point when it is no longer required to provide services

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

1. Organization and Summary of Significant Accounting Policies (continued)

to that patient, which is generally at the time of discharge or the completion of the outpatient visit. Substantially all of its performance obligations relate to contracts with a duration of less than one year. Unsatisfied or partially unsatisfied performance obligations primarily relate to inpatient acute care services at the end of the reporting period for patients who remain admitted at that time (in-house patients). The performance obligations for in-house patients are generally completed when the patients are discharged, which for the majority of the Hospital's in-house patients occurs within days or weeks after the end of the reporting period.

Net patient service revenue for the three months ended March 31, 2025 and 2024 by payor is as follows (in thousands):

	2025	March 31, 2024
	<i>(Unaudited)</i>	
Medicare ¹	\$ 802,255	\$ 773,016
Medicaid ¹	453,741	443,725
Commercial carriers and health maintenance organizations	1,651,318	1,639,400
Self-pay	37,669	28,564
	\$ 2,944,983	\$ 2,884,705

¹ Amounts are inclusive of Medicare and Medicaid program activity administered by managed care organizations.

Deductibles, copayments and coinsurance under third-party payment programs which are the patient's responsibility are included within the third-party payors amounts above.

Subsequent changes to the estimate of the transaction price (determined on a portfolio basis when applicable) are generally recorded as adjustments to patient service revenue in the period of the change. For the three months ended March 31, 2025 and 2024, changes in the Hospital's estimates of implicit price concessions, discounts, contractual adjustments or other reductions to expected payments for performance obligations satisfied in prior periods were not significant. Portfolio collection estimates are updated quarterly based on collection trends. Subsequent changes that are determined to be the result of an adverse change in the patient's ability to pay based on current or future estimated credit losses (determined on a portfolio basis when applicable) are recorded as bad debt expense. Bad debt expense for the three months ended March 31, 2025 and 2024 was not significant.

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

1. Organization and Summary of Significant Accounting Policies (continued)

At March 31, 2025 and December 31, 2024, accounts receivable is comprised of the following components (in thousands):

	March 31, 2025	December 31, 2024
	<i>(Unaudited)</i>	<i>(Audited)</i>
Patient receivables	\$ 1,632,597	\$ 1,691,682
Contract assets	72,953	64,983
	\$ 1,705,550	\$ 1,756,665

Contract assets are related to in-house patients who were provided services during the reporting period but were not discharged as of the reporting date and for which the Hospital may not have the right to bill.

The Hospital has established estimates, based on information presently available, of amounts due to or from Medicare and non-Medicare payors for adjustments to current and prior years' payment rates, based on industry-wide and Hospital-specific data. Medicare cost reports, which serve as the basis for final settlement with the Medicare program, have been audited by the Medicare fiscal intermediary and settled through 2020 for NYPH and NYP/Queens, and through 2021 for NYP/Hudson Valley, although revisions to final settlements or other retroactive changes could be made. Other years and various issues remain open for Medicare audit and settlement, as are numerous issues related to the New York State Medicaid program for prior years. As a result, there is at least a reasonable possibility that recorded estimates will change by a material amount when open years are settled, audits are completed, and additional information is obtained.

Settlements with third-party payors for cost report filings and retroactive adjustments due to ongoing and future audits, reviews or investigations are considered variable consideration and are included in the determination of the estimated transaction price for providing patient care. These settlements are estimated based on the terms of the payment agreement with the payor, correspondence from the payor and the Hospital's historical settlement activity (for example, cost report final settlements or repayments related to recovery audits), including an assessment to ensure that it is probable that a significant reversal in the amount of cumulative revenue recognized will not occur when the uncertainty associated with the retroactive adjustment is subsequently resolved. Such estimates are determined through either a probability-weighted estimate or an estimate of the most likely amount, depending on the circumstances related to a given estimated settlement item. Estimated settlements are adjusted in future periods as adjustments become known (that is, new information becomes available), or as years are settled or are no longer subject to such audits, reviews, and investigations. For the three months ended March 31, 2025 and 2024, the net effect of the Hospital's revisions to prior year estimates resulted in net patient service revenue increasing by approximately \$22.5 million and \$52.5 million, respectively.

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

1. Organization and Summary of Significant Accounting Policies (continued)

The current Medicaid, Medicare and other third-party payor programs are based upon extremely complex laws and regulations that are subject to interpretation. Noncompliance with such laws and regulations could result in fines, penalties and exclusion from such programs. The Hospital is not aware of any allegations of non-compliance that could have a material adverse effect on the accompanying consolidated financial statements and believes that it is in compliance in all material respects with all applicable laws and regulations.

There are various proposals at the federal and state levels that could, among other things, significantly reduce payment rates or modify payment methods. The ultimate outcome of these proposals and other market changes, including the potential effects of or revisions to health care reform that has been or will be enacted by the federal and state governments, cannot be determined presently. Future changes in the Medicare and Medicaid programs and any reduction of funding could have an adverse impact on the Hospital. Additionally, certain payors' payment rates for various years have been appealed by the Hospital. If the appeals are successful, additional income applicable to those years could be realized.

Investments

Investments are held in the Hospital's short-term and assets limited as to use portfolios. All investments are classified as trading investments and, excluding interests in common collective equity funds, alternative investments and pooled investments held by related parties, are carried at fair value determined as described in Note 5. Common collective equity funds and alternative investments are reported in the accompanying unaudited consolidated statements of financial position based upon net asset values derived from the application of the equity method of accounting.

Alternative investment interests generally are structured such that the Hospital (or its related party investment pool) holds a limited partnership interest or an interest in an investment management company, including hedge funds, private equity funds and private real estate funds. The Hospital's ownership structure does not provide for control over the related investees and the Hospital's financial risk is limited to the carrying amount reported for each investee, in addition to any unfunded capital commitment.

Individual investment holdings within the alternative investments include non-marketable and market-traded debt, equity and real asset securities and interests in other alternative investments. The Hospital may be exposed indirectly to securities lending, short sales of securities and trading in futures and forward contracts, options and other derivative products. Alternative investments often have liquidity restrictions under which the Hospital's capital may be divested only at specified times.

Financial information used by the Hospital to evaluate its alternative investments is provided by the investment manager or general partner and includes fair value valuations (quoted market prices and values determined through other means) of underlying securities and other financial instruments held by the investee and estimates that require varying degrees of judgment.

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

1. Organization and Summary of Significant Accounting Policies (continued)

The financial statements of the investee companies are audited annually by independent auditors, although the timing for reporting the results of such audits for certain investee companies does not coincide with the Hospital's annual financial statement reporting.

Interests in pooled investments held by related parties are reported based upon unitized net asset value (see Note 4). Assets held in the Hospital's defined benefit pension plans are carried at fair value. Fair value for alternative investments held by such plans is determined for each investment using net asset values as a practical expedient, as permitted by GAAP, rather than using another valuation method to independently estimate fair value.

There is uncertainty in determining fair values of alternative investments arising from factors such as lack of active markets (primary and secondary), lack of transparency into underlying holdings, time lags associated with reporting by the investee companies and the subjective evaluation of liquidity restrictions. As a result, there is at least a reasonable possibility that estimates will change in the near term.

Tax Status

The majority of the entities comprising the Hospital are Section 501(c)(3) organizations exempt from Federal income taxes on related income under Section 501(a) of the Internal Revenue Code (IRC). These entities are also exempt from New York State income taxes. NYPH, Community Programs, NYP/Queens, Kings Physician Services, P.C., and certain other entities are exempt from New York City income taxes. NICL is a captive insurance company domiciled in Bermuda with limited operations and no material income tax exposure.

Certain subsidiaries and controlled entities of the Hospital are for-profit entities. Taxable operations and the potential for income taxes from these entities and from unrelated business activities of the tax exempt entities are not significant to the accompanying unaudited consolidated financial statements.

2. Pension and Similar Benefits Plans

The Hospital provides pension and similar benefits to its employees through several plans, including participation in various multiemployer plans for certain union employees; a nonqualified defined benefit plan for certain NYPH executives (the Nonqualified Pension Plan); a defined benefit retirement plan under a collective bargaining agreement for certain employees of NYP/LMH (the NYP/LMH Pension Plan); and a defined benefit plan for certain eligible Hospital employees (NewYork-Presbyterian Plan) (the NYP/LMH Pension Plan and the NewYork-Presbyterian Plan are collectively referred to as the Pension Plans).

The Hospital funds the noncontributory defined benefit plans in accordance with the minimum funding requirements of the Employee Retirement Income Security Act of 1974, as amended (ERISA), plus additional amounts that the Hospital may deem appropriate from time to time. Amounts contributed to the Pension Plans are based on actuarial valuations.

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

2. Pension and Similar Benefits Plans (continued)

The Hospital currently does not expect to make contributions to the Pension Plans in 2025 (none in 2024); however, management will reevaluate the funding status periodically in 2025.

The Hospital contributed \$1.2 million and \$1.5 million to its other postretirement benefit plans during the three months ended March 31, 2025 and 2024, respectively. The Hospital expects to contribute approximately \$3.5 million to its other postretirement benefit plans for the remainder of 2025.

The Hospital recognizes in the accompanying consolidated statements of financial position an asset, for a defined benefit plan's overfunded status, or a liability, for a plan's underfunded status; measures a defined benefit plan's assets and obligations that determine funded status as of the end of the Hospital's fiscal year; and recognizes the periodic change in the funded status of a defined benefit plan as a component of changes in net assets without donor restrictions in the year in which the change occurs. Amounts that are recognized as a component of changes in net assets without donor restrictions will be subsequently recognized as net periodic benefit cost in future periods. On a quarterly basis, the Hospital's management has elected to determine an interim discount rate and estimates the fair value of the plan assets and reports the change in pension and postretirement benefit liabilities to be recognized in future periods. The Hospital's interim estimates are subject to volatility from market interest rates and amounts reported on an interim basis are subject to change and do not reflect the impact of other changes to the funded status of the defined benefit plans as measured at year end.

Net periodic benefit cost consists of the following for the three months ended March 31, 2025 and 2024 (in thousands):

	Pension Plans		Postretirement Benefit Plans	
	2025	2024	2025	2024
	<i>(Unaudited)</i>		<i>(Unaudited)</i>	
Service cost	\$ 35,605	\$ 28,960	\$ 1,998	\$ 2,179
Interest cost	26,401	31,269	580	617
Expected return on plan assets	(43,579)	(46,545)	—	—
Net amortization of prior service cost	—	—	(200)	(201)
Recognized actuarial losses (gains)	—	—	15	(10)
Net periodic pension cost and postretirement benefits cost	\$ 18,427	\$ 13,684	\$ 2,393	\$ 2,585

The amounts for 2025 in the table above are based on preliminary estimates determined by the Hospital's actuaries which will be finalized later in the year.

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

3. Professional Liability Insurance Program

In 1978, NYPH, in conjunction with a number of unrelated health care entities, participated in the formation of captive insurance companies (collectively, the Captive) to provide professional liability and general liability insurance to its participants. The premiums are based on a modified claims-made coverage and are actuarially determined based on the actual experience of the Captive, Hospital-specific experience, and estimated current exposure. The Captive has reinsurance coverage from reinsurers for certain amounts above its self-insured retained limits. The professional liability tower currently provides reinsurance coverage in excess of \$200.0 million in the aggregate annually to the Hospital and the other participating entities. In January 1996, rights to equity in the Captive were transferred to New York-Presbyterian Fund, Inc. (Fund, Inc.), a related party. Accordingly, insurance premiums are paid by the Hospital initially to Fund, Inc. (refer to Note 10 of the Hospital's audited consolidated financial statements for the years ended December 31, 2024 and 2023).

Effective July 1, 2013, August 1, 2014, February 1, 2015, December 1, 2015 and July 1, 2022, NewYork-Presbyterian Lower Manhattan Hospital (NYP/LMH), NYP Westchester (formerly NewYork-Presbyterian/Lawrence Hospital), NYP/Hudson Valley, NYP/Queens and NewYork-Presbyterian/Brooklyn Methodist (NYP Brooklyn Methodist), respectively, became insured by the Captive. Prior to each entity's respective effective date with the Captive, the entities were covered by various commercially purchased policies or self-insured programs. For hospital-liability claims incurred through June 30, 2022, NYP Brooklyn Methodist participated in a separate captive insurance program which included certain self-insured retentions; certain physician-liability coverage continues to be obtained through this separate captive insurance program subsequent to July 1, 2022. The notes to the audited consolidated financial statements of the Hospital for the years ended December 31, 2024 and 2023 include additional disclosures which describe the Hospital's accounting for its professional liability insurance program and related matters.

Effective January 1, 2024, NICL provides excess coverage for certain claims under a deposit accounting model as part of the Hospital's professional liability insurance program.

The Hospital's estimates for professional liabilities are based upon complex actuarial calculations which utilize factors such as historical claims experience for the Hospital and related industry factors, trending models, estimates for the payment patterns of future claims and present value discount factors. These estimates may also be impacted by recent or proposed changes to the statute of limitations impacting certain existing or unreported claims. As a result, there is at least a reasonable possibility that recorded estimates will change by a material amount in the near term (see Note 6). Revisions to estimated amounts resulting from actual experience differing from projected expectations are recorded in the period the information becomes known or when changes are anticipated.

4. Investments and Assets Limited as to Use

The Hospital's investments are either separately invested or held on a pooled basis with related parties (pooled with Fund, Inc. or included in unitized investment funds within Hudson East River Systems, LLC (HERS)). Fund, Inc., NYPH and the Regional Hospitals maintain member capital accounts with HERS. Each entity's member capital account for HERS is tracked for various strategic pools of investments. Each entity is allocated income or loss from investments on a monthly basis based on the underlying fair values and net asset values of the invested securities and their individual unitized capital

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

4. Investments and Assets Limited as to Use (continued)

interest. The Hospital recognizes in its accompanying consolidated statements of financial position only the unitized portion of the HERS assets attributable to the Hospital's member capital accounts. The HERS assets attributable to the Hospital, consisting of separate member capital accounts for NYPH, NYP/Hudson Valley and NYP/Queens, represented approximately 74% of the total HERS portfolio at March 31, 2025 and December 31, 2024; the composition of the HERS investment portfolio asset allocation is provided in this Note.

The composition and reported value of short-term investments at March 31, 2025 and December 31, 2024 consist of the following (in thousands):

	March 31, 2025	December 31, 2024
	<i>(Unaudited)</i>	<i>(Audited)</i>
Investments held by the Hospital:		
Marketable securities, carried at fair value	\$ 200,810	\$ 199,712
Hedge funds, accounted for using the equity method	2,012	1,981
Total investments held by the Hospital	202,822	201,693
Unitized investments held by HERS, accounted for using the equity method	2,539,145	2,518,604
Pooled investments held by Fund, Inc. on behalf of NYP/Queens, accounted for using the equity method	10,164	10,190
Total short-term investments	\$ 2,752,131	\$ 2,730,487

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

4. Investments and Assets Limited as to Use (continued)

The composition and reported value of assets limited as to use, which excludes the beneficial interest in net assets held by related organizations, at March 31, 2025 and December 31, 2024 consist of the following (in thousands):

	March 31, 2025	December 31, 2024
	<i>(Unaudited)</i>	<i>(Audited)</i>
Investments accounted for at fair value:		
Marketable securities, carried at fair value	\$ 150,329	\$ 111,848
Investments held by captive insurance companies, at allocated fair value	287,756	287,756
Total investments accounted for at fair value	438,085	399,604
 Hospital held investments accounted for using the equity method:		
Common collective equity funds	38,266	38,038
Hedge funds	186	223
Private equity	866	807
Private real assets	25	25
	39,343	39,093
Pooled investments held by Fund, Inc. on behalf of NYP/Queens, accounted for using the equity method	28,318	28,388
Unitized investments held by HERS, accounted for using the equity method	6,627,861	6,638,030
Total investments accounted for using the equity method	6,695,522	6,705,511
 Total assets limited as to use	7,133,607	7,105,115
Less current portion	56,609	55,342
Assets limited as to use – noncurrent	\$ 7,076,998	\$ 7,049,773

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

4. Investments and Assets Limited as to Use (continued)

The asset allocation of the HERS investment portfolio and unfunded commitments and liquidity restrictions for certain alternative investments held by HERS at March 31, 2025 are as follows (in thousands):

Description of Investment	Asset Allocation	Unfunded Commitments	Redemption Frequency (if Currently Eligible)	Redemption Notice Period
<i>(Unaudited)</i>				
Cash and cash equivalents	2%			
Fixed income securities:				
U.S. government bonds and notes	7			
Mortgage and asset-backed	6			
Corporate	5			
Equities:				
U.S. equities	1			
Non-U.S. equities	1			
Common collective equity funds	38	\$ 63,414	Weekly to annually	5 to 150 days
Hedge funds	15	239,125	Monthly to annually	30 to 180 days
Private equity	17	1,781,487	*	*
Private real assets	8	515,905	*	*
	100%	\$ 2,599,931		

* HERS' liquidity restrictions range from several months to seven plus years for certain private equity and private real asset investments depending upon the investment's fund cycle. Liquidity restrictions may apply to all or portions of a particular invested amount.

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

5. Fair Value Measurements

The Hospital uses various methods of calculating fair value of its financial assets and liabilities, when applicable. The Hospital defines fair value as the price that would be received to sell an asset or paid to transfer a liability in an orderly transaction between market participants at the measurement date and establishes a framework for measuring fair value. Fair value measurements are applied based on the unit of account from the Hospital's perspective. The unit of account determines what is being measured by reference to the level at which the asset or liability is aggregated (or disaggregated).

The Hospital uses a three-level valuation hierarchy for disclosure of fair value measurements. The valuation hierarchy is based upon the transparency of inputs to the valuation of an asset or liability as of the measurement date. The three levels are defined as follows:

Level 1: Quoted prices (unadjusted) in active markets that are accessible at the measurement date for identical assets or liabilities. The fair value hierarchy gives the highest priority to Level 1 inputs.

Level 2: Observable inputs that are based on inputs not quoted in active markets but corroborated by market data.

Level 3: Unobservable inputs are used when little or no market data is available. The fair value hierarchy gives the lowest priority to Level 3 inputs.

A financial instrument's categorization within the valuation hierarchy is based upon the lowest level of input that is significant to the fair value measurement. In determining fair value, the Hospital uses valuation techniques that maximize the use of observable inputs and minimize the use of unobservable inputs to the extent possible and considers non-performance risk in its assessment of fair value. As described in Note 1, the Hospital records certain investments at fair value based on net asset value.

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

5. Fair Value Measurements (continued)

The following tables present financial instruments carried at fair value, excluding assets invested in the Pension Plans, as of March 31, 2025 and December 31, 2024 (in thousands):

March 31, 2025 (Unaudited)				
	Total	Level 1	Level 2	Level 3
Cash and cash equivalents – held for investment	\$ 121,810	\$ 121,810	\$ –	\$ –
Fixed income:				
U.S. government	92,029	92,029	–	–
Corporate	324	–	324	–
Equities:				
U.S. equities ^(a)	48,240	48,240	–	–
Non-U.S. equities ^(b)	28,117	28,117	–	–
Mutual funds	60,619	60,619	–	–
Investments held by captive insurance companies ^(c)	287,756	–	230,548	57,208
	<u>\$ 638,895</u>	<u>\$ 350,815</u>	<u>\$ 230,872</u>	<u>\$ 57,208</u>

December 31, 2024 (Audited)				
	Total	Level 1	Level 2	Level 3
Cash and cash equivalents – held for investment	\$ 81,327	\$ 81,327	\$ –	\$ –
Fixed income:				
U.S. government	92,460	92,460	–	–
Corporate	318	–	318	–
Equities:				
U.S. equities ^(a)	50,677	50,677	–	–
Non-U.S. equities ^(b)	26,744	26,744	–	–
Mutual funds	60,034	60,034	–	–
Investments held by captive insurance companies ^(c)	287,756	–	230,548	57,208
	<u>\$ 599,316</u>	<u>\$ 311,242</u>	<u>\$ 230,866</u>	<u>\$ 57,208</u>

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

5. Fair Value Measurements (continued)

The Hospital's investments in common collective equity funds, alternative investments, unitized investments held by HERS and pooled investments held by Fund, Inc. are reported using the equity method of accounting and, therefore, are not included in the tables above (see Notes 1 and 4).

Financial instruments invested in the Hospital's Pension Plans at fair value are classified in the tables below in one of the three categories described above as of March 31, 2025 and December 31, 2024 (in thousands):

March 31, 2025 (Unaudited)				
	Total	Level 1	Level 2	Level 3
Cash and cash equivalents	\$ 33,135	\$ 33,135	\$ –	\$ –
Fixed income:				
U.S. government	218,015	218,015	–	–
Equities:				
U.S. equities ^(a)	65	65	–	–
Non-U.S. equities ^(b)	61	61	–	–
	<u>251,276</u>	<u>\$ 251,276</u>	<u>\$ –</u>	<u>\$ –</u>
Asset measured at net asset value as a practical expedient:				
Common collective equity funds ^(d)	693,903			
Hedge funds ^(e)	520,750			
Private equity ^(f)	656,936			
Private real assets ^(g)	406,649			
	<u>\$ 2,529,514</u>			

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

5. Fair Value Measurements (continued)

	December 31, 2024 <i>(Audited)</i>			
	Total	Level 1	Level 2	Level 3
Cash and cash equivalents	\$ 60,306	\$ 60,306	\$ —	\$ —
Fixed income:				
U.S. government	212,052	212,052	—	—
Equities:				
U.S. equities ^(a)	98	98	—	—
Non-U.S. equities ^(b)	58	58	—	—
	<u>272,514</u>	<u>\$ 272,514</u>	<u>\$ —</u>	<u>\$ —</u>
Asset measured at net asset value as practical expedient:				
Common collective equity funds ^(d)	708,500			
Hedge funds ^(e)	498,938			
Private equity ^(f)	655,636			
Private real assets ^(g)	<u>408,393</u>			
	<u>\$ 2,543,981</u>			

^(a) Equity portfolios invested in common stock of corporations primarily domiciled in the United States.

^(b) Equity portfolios invested in common stock of corporations primarily domiciled outside the United States, including emerging market countries.

^(c) Investments held by captive insurance companies consist of assets which are pooled with other assets maintained by the companies and include investments in marketable securities and alternative investments that are recorded by the captive insurance companies at fair value based on quoted market prices or other means for the companies' holdings of alternative investments. The Hospital reports an allocation of the fair value of the pooled investments in its accompanying consolidated statements of financial position.

^(d) Common collective equity funds invested in common stock of corporations domiciled in the United States and outside the United States, including emerging market countries.

^(e) Hedge funds include long and short equity, multi-strategy, event driven and relative value funds invested with managers who invest with different strategies and typically employ some

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

5. Fair Value Measurements (continued)

leverage. In long and short equity, fund managers create a portfolio of long positions in stocks expected to appreciate over time and short positions in stocks expected to depreciate. Event driven managers create a portfolio designed to profit from corporate events, such as mergers, spin-offs, defaults and bankruptcy. Relative value managers invest in long and short positions, but typically have a more neutral net market position than long and short. Multi-strategy is a fund employing a variety of hedge fund strategies.

- (f) Private equity investments include limited partnership investments in funds pursuing strategies in corporate buyouts, venture capital, growth equity, distressed and turnaround investments.
- (g) Real estate, infrastructure and natural resources investments.

The following is a description of the Hospital's valuation methodologies for assets measured at fair value. The fair value methodologies are not necessarily indicators of investment risk but are descriptive of the measures used to arrive at fair value pricing. Fair value for Level 1 is based upon quoted market prices. Investments classified as Level 2 are primarily valued using techniques that are consistent with the market approach. Valuations for Level 2 are based on quoted prices for similar instruments in active markets, quoted prices for identical or similar instruments in markets that are not active, and model-based valuation techniques for which all significant assumptions are observable in the market or can be corroborated by observable market data for substantially the full term of the assets. Inputs, which include broker/dealer quotes, reported/comparable trades, and benchmark yields are obtained from various sources including market participants, dealers and brokers. Level 3 assets consist of interests in pooled alternative investments managed by captive insurance companies, the valuation for alternative investments is described in Note 1. The Pension Plans' common collective equity funds and alternative investments are measured at net asset value; the valuation for these alternative investments is described in Note 1. The methods described above may produce a fair value that is not indicative of net realizable value or reflective of future fair values. Furthermore, while the Hospital believes its valuation methods are appropriate and consistent with other market participants, the use of different methodologies or assumptions to determine the fair value of certain financial instruments could result in a different estimate of fair value at the reporting date.

The New York and Presbyterian Hospital

Notes to Unaudited Consolidated Financial Statements (continued)

March 31, 2025

5. Fair Value Measurements (continued)

The following is a summary of investments (by major class) that have restrictions on the Hospital's or the Hospital's pension plans ability to redeem its investments at the measurement date, any unfunded capital commitments and the investments strategies of the investees as of March 31, 2025 (excluding unitized investments held by HERS and pooled investments held by Fund, Inc.; see Note 4) (in thousands):

<i>(Unaudited)</i>				
Description of Investment	March 31, 2025 Value	Unfunded Commitments	Redemption Frequency (If Currently Eligible)	Redemption Notice Period
Common collective equity funds	\$ 732,169	\$ —	Weekly to annually	5 to 150 days
Hedge funds	522,948	65,064	Monthly to annually	30 to 180 days
Private equity	657,802	315,128	*	*
Private real assets	406,674	157,982	*	*
	<u>\$ 2,319,593</u>	<u>\$ 538,174</u>		

* Liquidity restrictions range from several months to seven plus years for certain private equity and private real asset investments depending upon the investments' fund cycle. Liquidity restrictions may apply to all or portions of a particular invested amount.

6. Commitments and Contingencies

Various lawsuits and claims arising in the normal course of operations are pending or are in progress against the Hospital. Such lawsuits and claims are specifically covered by insurance, are addressed through management-recorded provisions for certain estimated losses based on presently known information to the extent they are not covered by insurance or are not deemed material.

Litigation pending against the Hospital includes, among other matters, certain cases in which the Hospital is a defendant involving alleged sexual abuse by a former faculty member of each School or a physician employed by NYP/Queens. These cases continue to receive media attention. While the Hospital and the Schools have been named as defendants in each respective case involving a former faculty member, the Hospital does not necessarily have responsibility or liability for the acts of a physician employed by one of the Schools.

Additionally, the Hospital and the Schools, as applicable to each School's respective case, are presently negotiating certain verifiable claims and evaluating the veracity of other alleged claims arising from these allegations. Due to the enactment in 2022 of the New York State Adult Survivors Act (ASA) and the New York City Gender Motivated Violence Act (GMVA), a high volume of claims related to these matters were filed in late 2023 prior to the end of the lookback period for the ASA (November 23, 2023). Additional claims were recently filed as the GMVA two-year lookback period, which began on March 1, 2023, ended on March 1, 2025. The majority of the more recently filed claims have not been fully evaluated to assess the validity of the claims.

The New York and Presbyterian Hospital
Notes to Unaudited Consolidated Financial Statements (continued)
March 31, 2025

6. Commitments and Contingencies (continued)

While the direct financial impact on the Hospital from certain of these matters cannot be determined at this time, based on presently known information and estimates recorded within the accompanying financial statements, which reflect the Hospital's experience with settled claims, advice from the Hospital's legal counsel, information provided by attorneys purporting to represent additional claimants, and the terms of other agreements related to these matters, management believes that any further loss which may arise from these actions will not have a material adverse effect on the financial position or results of operations of the Hospital. As these physician matters have developed materially in recent years, the Hospital has recorded an estimate for potential losses at March 31, 2025 and December 31, 2024. The estimates have been developed based on presently known information, as described above, and considering estimation factors to reflect possible outcomes of contingencies for the ultimate amount of available insurance or reinsurance, the circumstances that may have an impact on apportionment of liability, case settlement terms, estimates for claims incurred but not reported, and other unresolved contingencies. Additional losses in excess of the amount accrued based on presently known information at March 31, 2025 and December 31, 2024 are reasonably possible as such contingencies are resolved. Accordingly, there is at least a reasonable possibility that recorded estimates will change by a material amount in the near term.

Refer to historical disclosure in Note 12 to the audited consolidated financial statements for the years ended December 31, 2024 and 2023 for its annual disclosure of other matters involving commitments and contingencies.

7. Other Revenue

Other revenue consisted of the following for the nine months ended March 31, 2025 and 2024 (in thousands):

	March 31,	
	2025	2024
Recurring other revenue	\$ 428,316	\$ 339,562
FEMA Disaster Relief Fund – operating expenses	30,068	–
	<u>\$ 458,384</u>	<u>\$ 339,562</u>

8. Subsequent Events

Subsequent events have been evaluated through May 23, 2025, which is the date the unaudited consolidated financial statements were issued. No events have occurred that require disclosure in or adjustment to the accompanying unaudited consolidated financial statements.

The New York and Presbyterian Hospital
Unaudited Consolidating Statement of Financial Position
Three Months Ended March 31, 2025
(In Thousands)

Unaudited consolidating Information

The following table summarizes the unaudited consolidating statement of financial position at March 31, 2025 (in thousands):

	<u>Obligated Group</u>	<u>Network</u>	<u>NYP Medical</u>	<u>NYP Medical</u>	<u>NYP Community Programs, Inc.</u>		<u>Eliminations /</u>	<u>Consolidated</u>
	<u>NYPH</u>	<u>Insurance</u>	<u>Group/Brooklyn</u>	<u>Group/Westchester</u>	<u>NYP/Hudson</u>	<u>NYP/Queens</u>	<u>Reclassifications</u>	
		<u>Company Ltd.</u>	<u>Entities</u>	<u>and Other Entities</u>	<u>Valley</u>			
	(Unaudited)							
Assets								
Current assets:								
Cash, cash equivalents and short-term investments:								
Cash and cash equivalents	\$ 502,916	\$ 5,200	\$ 4,003	\$ 57,153	\$ 23,551	\$ 33,082	\$ —	\$ 625,905
Short-term investments	2,645,392	—	—	—	34,378	72,361	—	2,752,131
Total cash, cash equivalents and short-term investments	3,148,308	5,200	4,003	57,153	57,929	105,443	—	3,378,036
Patient accounts receivable – net	1,481,430	—	10,367	9,491	54,168	150,094	—	1,705,550
Other current assets	569,419	36	1,024	3,780	15,174	138,994	—	728,427
Assets limited as to use – current portion	54,831	643	—	—	—	1,135	—	56,609
Professional liabilities insurance recoveries receivable and related deposits – current portion	86,442	—	—	—	4,201	10,720	—	101,363
Beneficial interest in net assets held by related organizations – current portion	73,141	—	—	—	—	—	—	73,141
Due from related organizations – net	507,472	—	43,905	—	—	—	(503,276)	48,101
Loans receivable from Regional Hospitals – current portion	9,327	—	—	—	—	—	(9,327)	—
Total current assets	5,930,370	5,879	59,299	70,424	131,472	406,386	(512,603)	6,091,227
Assets limited as to use – noncurrent	6,993,846	—	—	18,450	16,920	47,782	—	7,076,998
Property, buildings and equipment – net	4,747,393	—	16,977	10,423	167,383	378,604	—	5,320,780
Operating lease assets	389,966	—	—	10,791	28,593	39,526	—	468,876
Other noncurrent assets – net	178,491	—	—	—	7,733	21,179	—	207,403
Loans receivable from Regional Hospitals – noncurrent	180,498	—	—	—	—	—	(180,498)	—
Professional liabilities insurance recoveries receivable and related deposits – noncurrent	347,470	—	—	—	19,920	48,247	—	415,637
Beneficial interest in net assets held by related organizations – noncurrent	3,051,437	—	—	—	—	—	—	3,051,437
Total assets	\$ 21,819,471	\$ 5,879	\$ 76,276	\$ 110,088	\$ 372,021	\$ 941,724	\$ (693,101)	\$ 22,632,358

See accompanying note to supplementary information.

The New York and Presbyterian Hospital
Unaudited Consolidating Statement of Financial Position (continued)

Three Months Ended March 31, 2025
(In Thousands)

Unaudited consolidating Information (continued)

	Obligated Group NYPH	Network Insurance Company Ltd.	NYP Medical Group/Brooklyn Entities	NYP Medical Group/Westchester and Other Entities	NYP Community Programs, Inc.		Eliminations / Reclassifications	Consolidated
					NYP/Hudson Valley	NYP/Queens		
(Unaudited)								
Liabilities and net assets								
Current liabilities:								
Long-term debt – current portion	\$ 74,959	\$ –	\$ –	\$ –	\$ 934	\$ 1,404	\$ –	\$ 77,297
Loans payable to NYPH – current portion	–	–	–	–	2,860	6,467	(9,327)	–
Operating lease liability – current portion	42,596	–	–	2,251	3,544	9,545	–	57,936
Accounts payable and accrued expenses	823,379	710	16,496	51,819	49,798	134,206	970	1,077,378
Accrued salaries and related liabilities	678,545	–	27,347	7,342	38,483	32,214	–	783,931
Postretirement benefit liability – current portion	3,623	–	–	–	–	1,071	–	4,694
Professional and other insurance liabilities – current portion	104,705	–	–	–	4,209	11,564	–	120,478
Other current liabilities	529,645	–	7,838	2,084	–	–	–	539,567
Due to related organizations – net	–	–	–	36,177	110,459	357,610	(504,246)	–
Total current liabilities	2,257,452	710	51,681	99,673	210,287	554,081	(512,603)	2,661,281
Long-term debt	3,749,633	–	–	–	3,140	2,201	–	3,754,974
Loans payable to NYPH	–	–	–	–	31,235	149,263	(180,498)	–
Operating lease liability	370,682	–	–	8,979	25,304	32,277	–	437,242
Professional and other insurance liabilities	851,729	443	–	12,521	28,334	84,410	–	977,437
Postretirement benefit liability	26,630	–	–	–	–	15,959	–	42,589
Other noncurrent liabilities	462,585	–	362	308	9,714	12,432	–	485,401
Total liabilities	7,718,711	1,153	52,043	121,481	308,014	850,623	(693,101)	8,358,924
Net assets:								
Net assets without donor restrictions	10,962,506	4,726	24,233	(11,808)	58,124	71,572	–	11,109,353
Net assets with donor restrictions	3,138,254	–	–	415	5,883	19,529	–	3,164,081
Total net assets	14,100,760	4,726	24,233	(11,393)	64,007	91,101	–	14,273,434
Total liabilities and net assets	\$ 21,819,471	\$ 5,879	\$ 76,276	\$ 110,088	\$ 372,021	\$ 941,724	\$ (693,101)	\$ 22,632,358

See accompanying note to supplementary information.

The New York and Presbyterian Hospital
Unaudited Consolidating Statement of Operations

Three Months Ended March 31, 2025

(In Thousands)

Unaudited consolidating Information (continued)

	<u>Obligated Group</u>	Network	NYP Medical	NYP Medical	<u>NYP Community Programs, Inc.</u>		Eliminations /	
	<u>NYPH</u>	<u>Insurance</u>	<u>Group/Brooklyn</u>	<u>Group/Westchester</u>	<u>NYP/Hudson</u>	<u>NYP/Queens</u>	<u>Reclassifications</u>	<u>Consolidated</u>
		<u>Company Ltd.</u>	<u>Entities</u>	<u>and Other Entities</u>	<u>Valley</u>			
					(Unaudited)			
Operating revenues								
Net patient service revenue	\$ 2,514,806	\$ —	\$ 39,748	\$ 16,514	\$ 87,252	\$ 286,663	\$ —	\$ 2,944,983
Other revenue	197,898	—	41,199	13,727	5,098	254,623	(54,161)	458,384
Total operating revenues	2,712,704	—	80,947	30,241	92,350	541,286	(54,161)	3,403,367
Operating expenses								
Salaries and wages	1,222,901	—	52,958	18,773	51,081	162,623	(129)	1,508,207
Employee benefits	324,462	—	10,025	5,700	13,582	50,413	(18)	404,164
Supplies and other expenses	900,359	—	17,461	5,878	30,756	319,555	(54,014)	1,219,995
Interest expense, net of amortization	34,475	—	—	—	398	1,134	—	36,007
Depreciation and amortization	133,626	—	551	362	3,587	10,705	—	148,831
Total operating expenses	2,615,823	—	80,995	30,713	99,404	544,430	(54,161)	3,317,204
Operating income (loss)	96,881	—	(48)	(472)	(7,054)	(3,144)	—	86,163
Investment return, net	14,094	—	48	366	525	298	—	15,331
Expected return on plan assets and other components of net periodic pension and postretirement cost	15,450	—	—	—	—	1,333	—	16,783
Excess (deficiency) of revenues over expenses	126,425	—	—	(106)	(6,529)	(1,513)	—	118,277
Other changes in net assets without donor restrictions:								
Net assets transfer to related parties	(3,712)	—	—	—	—	—	—	(3,712)
Other changes in net assets	(3)	—	—	(137)	—	—	—	(140)
Distributions from New York-Presbyterian Fund, Inc. for the purchase of fixed assets	25,146	—	—	—	—	—	—	25,146
Change in pension and postretirement benefit to be recognized in future periods	(27,531)	—	—	—	—	—	—	(27,531)
Change in net assets without donor restrictions	\$ 120,325	\$ —	\$ —	\$ (243)	\$ (6,529)	\$ (1,513)	\$ —	\$ 112,040

See accompanying note to supplementary information.

The New York and Presbyterian Hospital

Note to Unaudited Consolidating Supplementary Information

March 31, 2025

Basis of Presentation

The accompanying unaudited consolidating financial information includes the accounts of The New York and Presbyterian Hospital, NYP Community Programs, Inc. and other consolidated entities as described below. Refer to Note 1 to the unaudited consolidated financial statements in the previous section for additional information regarding the organizational structure, defined terms and reporting entity. The unaudited consolidating financial information includes columns for the following groups of entities with related activities based on corporate relationships, geography and/or purpose:

- **Network Insurance Company Ltd.**, Bermuda-based captive insurance company.
- **NYP Medical Group/Brooklyn Entities**, consisting of: Kings Physician Services, P.C.; Park Slope Medical Service, P.C.; Park Slope Pathology Services, P.C.; Park Slope Physician Services, P.C.; Park Slope Obstetrics Gynecology, P.C.; Park Slope Hematology & Oncology, P.C.; Park Slope Pediatric Medicine, P.C.; Park Slope Medicine, P.C.; Brooklyn Foot & Ankle, P.C.; Park Slope Emergency Physician Services, P.C.; Brooklyn Dental Services, P.C.; Brooklyn Radiology Services, P.C.; Park Slope Medical Health Provider, P.C.; and MSO of Kings County, LLC.
- **NYP Medical Group/Westchester and Other Entities**, consisting of: Lawrence Medical Associates, P.C. (d/b/a NewYork-Presbyterian Medical Group/Westchester); Lawrence Care, Inc., the sole corporate member of Lawrence Community Health Services; and NYP Sports Performance, LLC.
- **NYP/Hudson Valley**, consisting of: Hudson Valley Hospital Center; The Foundation of NYP/Hudson Valley Hospital; Westchester Putnam Health Management System, Inc.; Westchester Medical Practice, P.C. (d/b/a NewYork-Presbyterian Medical Group/Hudson Valley); Hudson Valley Ventures, Inc.; A.C. Ventures, Inc.; KNOWA Ventures, Inc.; and GI Ventures, Inc.
- **NYP/Queens**, consisting of: NewYork-Presbyterian/Queens; New York Queens Medicine and Surgery, PC (d/b/a NewYork-Presbyterian Medical Group/Queens); CRT Surgical Associates, PC (CRT); and New York Queens Charter Ventures, Inc. CRT is inactive.

The accompanying unaudited financial information has been prepared in accordance with accounting principles generally accepted in the United States of America. The notes to the unaudited consolidated financial statements in the previous section include disclosures regarding significant accounting policies. All significant intercompany balances and transactions have been eliminated in consolidation. Net asset transfers (to) from related parties in the consolidating statement of operations include distributions from NYPH to Hudson Valley, Queens and other related entities for the purchase of fixed assets.